# Voiding Function & Dysfunction Rapid Revision

Campbell-Walsh-Wein · Chapters 102–104, 106, 107, 109–113, 115, 123

For MCh Urology theory & viva · NEET-SS / competitive MCQs

**How to use (total ≈ 6h 03min even at a slow, careful pace)**

- Each chapter reads top-to-bottom in the order you would write an essay answer: definition → pathology → diagnosis → management → complications → follow-up. The same material serves theory and viva.
- Boxed **ALGORITHM** and shaded **KEY POINT** panels carry the reasoning examiners ask about ("why do we…?").
- **MCQ one-liners** end each chapter: re-read these the day before the paper.
- The last section puts the big treatment choices side by side for essay questions.

**Conventions**: **underlined bold** = number to memorise · MCQ = exam trivia only · → *Ch 107* = cross-reference · † = not in the source chapter notes (added because it is essential in practice). Abbreviations are spelled out at first use in each chapter.


| Chapter | Topic | Slow-reader time |
|---|---|---|
| 102 | Surgical, Radiographic and Endoscopic Anatomy of the Male Pelvis | 28 min |
| 103 | Physiology and Pharmacology of the Bladder and Urethra | 51 min |
| 104 | Pathophysiology and Classification of LUT Dysfunction | 16 min |
| 106 | Evaluation and Management of Males With Urinary Incontinence | 21 min |
| 107 | Urodynamic and Videourodynamic Evaluation of the LUT | 41 min |
| 109 | Neuromuscular Dysfunction of the Lower Urinary Tract | 42 min |
| 110 | Overactive Bladder | 24 min |
| 111 | Underactive Detrusor | 23 min |
| 112 | Nocturia | 47 min |
| 113 | Pharmacologic Management of Storage and Emptying Failure | 19 min |
| 115 | Electrical Stimulation and Neuromodulation | 18 min |
| 123 | Surgical Procedures for Sphincteric Incontinence in the Male | 25 min |
| ★ | Putting it together: voiding dysfunction decisions | 8 min |
| | **Total (25455 words, at 70 words/min)** | **6h 03min** |

# 102 Surgical, Radiographic and Endoscopic Anatomy of the Male Pelvis

*Reading time ≈ 28 min (slow pace)*

## 1 · BONY PELVIS

- Sacrum + paired innominate bones, each = **ilium, ischium, pubis** fused at the acetabulum. Sacroiliac (SI) joints posteriorly; cartilaginous symphysis anteriorly.
- **Sacrotuberous** (posterior; → ischial tuberosity) and **sacrospinous** (anterior; → ischial spine) ligaments limit upward sacral movement, giving the SI joint resilience under sudden load, and create the sciatic foramina: sacrospinous bisects the sciatic foramen into greater and lesser.
- Pelvic brim at **50–60°** to horizontal. **Arcuate line** (promontory → pubis) separates false pelvis (iliac fossae) from true pelvis (urogenital organs).
- Outlet: lower symphysis, ischiopubic rami, ischial tuberosities, sacrotuberous ligaments, coccyx. Iliac crest at **L4**.
- **Cooper's (pectineal) ligament** takes sutures in hernia repair and urethral suspension.

## 2 · LOWER ABDOMINAL WALL

### Fascia and extravasation

- Umbilicus: **T10** dermatome, usually at L3.
- **Camper's** (superficial fatty) carries the superficial inferior epigastric, superficial circumflex iliac and external pudendal vessels (femoral branches) — met at inguinal incisions and trocar sites. **Scarpa's** (deep membranous) fuses with fascia lata ~1 cm below the inguinal ligament and continues medially as **Colles fascia** → dartos.
- Colles fascia fuses to the ischiopubic rami and posterior edge of the perineal membrane. Hence after **anterior urethral injury** blood and urine collect in penis and scrotum, limited laterally → **"butterfly" perineal haematoma**; spread to thigh or buttock is blocked, but spread **upward** deep to Scarpa's continues up the abdominal wall to flank and back. Same fusions confine necrotising scrotal infection.
- †With Buck's fascia intact, the haematoma stays on the penile shaft; the butterfly pattern means Buck's fascia is breached.

### Rectus sheath

| | Above arcuate line | Below arcuate line |
|---|---|---|
| Anterior | External oblique + part of internal oblique | All three aponeuroses |
| Posterior | Rest of internal oblique + transversus | **Absent** — transversalis fascia + peritoneum only |

- Arcuate line ≈ one-third of the way from umbilicus to pubic crest; aponeuroses meet at the linea alba.
- Rectus intersections are tethered to the anterior sheath → **transverse division without retraction** is safe. Segmental nerves enter **laterally** → a paramedian incision lateral to rectus causes atrophy and ventral hernia.
- Pyramidalis: absent in ~20%.

### Inguinal canal

- ~**4 cm**; transmits **spermatic cord and ilioinguinal nerve**. Deep ring (transversalis fascia) lies midway between anterior superior iliac spine (ASIS) and pubic tubercle, **lateral to inferior epigastric vessels**; superficial ring (external oblique) superolateral to pubic tubercle.
- Walls: anterior — external oblique; posterior — transversalis fascia + conjoint tendon medially; roof — arching internal oblique/transversus; floor — iliopubic tract, inguinal ligament, lacunar ligament.

Contraction of the roof against the floor = **shutter mechanism**.

- Direct hernia **medial**, indirect **lateral** to inferior epigastric vessels.
- **Radical orchidectomy**: ligate cord at the internal ring → stump retracts into retroperitoneum.

**Peritoneal folds (laparoscopic map)**

| Fold (medial → lateral) | Contains | Significance |
|---|---|---|
| Median | Urachus | Bladder apex → umbilicus |
| Medial (paired) | Obliterated umbilical arteries | Incise **lateral** to them → obturator fossa, retropubic space |
| Lateral (paired) | Inferior epigastric vessels | Deep ring; hernia type |

## 3 · PELVIC FLOOR AND FASCIA

- Obturator internus fascia thickens into the **tendinous arch of levator ani** (ischial spine → pubis), origin of the pelvic diaphragm = levator ani + coccygeus.
- Levator ani: puborectalis (most anterior, U-sling behind anorectal junction — **faecal continence**), pubococcygeus, iliococcygeus. Somatic, **tonically contracted**, type I and II fibres; relaxes to void, contracts against Valsalva. **Kegel exercises** augment this tone after pelvic surgery.
- **Parietal fascia** lines walls (continuous with transversalis); **visceral fascia** sheaths organs; lateral organ attachments = **endopelvic fascia**, carrying all pelvic vessels to the organs — entering it is the gateway to the prostate.
- **Arcus tendineus fascia pelvis**: puboprostatic ligament → ischial spine; junction of parietal and visceral fascia; anchors urethral and bladder supports.

## 4 · PERINEUM

- Diamond (symphysis, ischiopubic rami, ischial tuberosities, sacrotuberous ligaments, coccyx); a line between the tuberosities divides **urogenital** and **anal** triangles.
- **Perineal membrane**: tough triangle across the pubic arch, perforated by the urethra; supports the sphincter; genitalia attach below.
- **Perineal body**: midline fibromuscular keystone into which nearly every pelvic muscle and fascia inserts; blends with perineal membrane and Denonvilliers fascia. Injury at perineal prostatectomy → urinary and faecal incontinence.
- **Ischiorectal fossa**: fat between levator ani and obturator internus; communicates with the pelvis via sciatic foramina → perirectal infection spreads into the pelvis.
- Anal sphincters: internal — smooth, sympathetic tone, upper two-thirds; external — voluntary, pudendal (rectal branches), lower two-thirds, deep part blends with puborectalis.
- Perineal prostatectomy MCQ: **Young** divides the central tendon, leaving external sphincter on the rectum; **Belt** passes **under** the external sphincter on the anterior rectal wall.
- Superficial pouch (deep to Colles): crura on ischiopubic rami with ischiocavernosus; bulb on perineal membrane with bulbospongiosus — both compress corpora in erection.
- **Internal pudendal artery**: out via greater, back via lesser sciatic foramen → **Alcock's canal** → **dorsal artery of penis**. Its veins join the dorsal venous complex via a lateral prostatic plexus → bleeding at prostatic dissection.
- **Pudendal nerve (S2–S4)**: external anal sphincter; striated urethral sphincter, levator ani, ischiocavernosus, bulbospongiosus; posterior scrotum; ends as dorsal nerve of penis.
- Penile length: **stretched ≈ erect length**; ranges are wide and population-dependent; shorter in erectile dysfunction (ED).

## 5 · PELVIC VESSELS AND LYMPHATICS

- Common iliac arteries (anterior to veins) divide at the **SI joint**. Middle sacral artery met in presacral dissection.
- **External iliac** → inferior epigastric (cremasteric, pubic branches) and deep circumflex iliac. **Accessory obturator vessels** from the inferior epigastric enter the obturator fossa — a pelvic lymph node dissection (PLND) hazard.
- **Internal iliac**: posterior trunk parietal (superior gluteal, ascending lumbar, lateral sacral); anterior trunk visceral (umbilical, obturator, middle rectal, inferior vesical, internal pudendal, inferior gluteal).
- Patent proximal umbilical artery gives the **superior vesical artery** and artery of the vas (joins cremasteric and testicular → collateral testicular supply). **Inferior vesical** (not internal pudendal) supplies the neurovascular bundle. Obturator artery lies deep and medial to the obturator nerve.
- External and internal iliac veins join **behind the internal iliac artery** — heavy bleeding if injured at PLND.
- **Dorsal venous complex (DVC)**: enters beneath symphysis anterior to the striated sphincter; ligated at radical prostatectomy, but ligation can damage the sphincter, especially laterally. **Santorini's plexus** met on entering lateral prostatic fascia.
- Lymph: frequent **right–left cross-drainage**. External iliac nodes — bladder, urachus, abdominal wall; internal iliac — prostate, bladder (obturator and presacral drain into them). **Node of Cloquet** (femoral canal) = **distal limit of PLND**.

## 6 · PELVIC NERVES

| Nerve | Roots | Key point |
|---|---|---|
| Ilioinguinal | L1 | Through inguinal canal → anterior scrotum |
| Genitofemoral | L1–L2 | On psoas — **lateral limit of PLND**; genital branch → cremaster, anterior scrotum |
| Femoral | L2–L4 | Within psoas; injured by psoas hitch, retractor blades, inguinal dissection |
| Obturator | L2–L4 | Seen at iliac bifurcation → obturator canal; PLND injury → **weak adduction** |
| Pudendal | S2–S4 | Perineal sensation; levator ani, urinary and anal sphincters |
| Nervi erigentes | S2–S4 | Parasympathetic to bladder, rectum |

> **KEY POINT** Anterior scrotum is L1 (ilioinguinal, genitofemoral); posterior scrotum is sacral (pudendal, posterior femoral cutaneous).

- **Sympathetic** (T10–L2) → superior hypogastric plexus → hypogastric nerves; **parasympathetic** (S2–S4) join them → **pelvic (inferior hypogastric) plexus**, beside the rectum centred on the seminal vesicles.
- Continues as **cavernosal nerves** posterolateral to prostate, **outside Denonvilliers fascia**; with prostatic vessels = **neurovascular bundle (NVB)** at **5 and 7 o'clock** at the apex. Injury at prostatectomy or rectal resection → ED.

## 7 · PELVIC VISCERA

### Rectum

- Begins at **S3**. Incising peritoneum of the **rectovesical pouch** exposes the seminal vesicles — the posterior, pelvic fascia-sparing robotic prostatectomy approach. Below it, **Denonvilliers fascia** separates rectum from prostate.
- Distal rectum near the apex = **commonest site of rectal injury** at prostatectomy.

### Bladder

- Extraperitoneal except the dome, which continues as the **urachus** → adenocarcinoma, fistula or cyst. Anterior **retropubic space** †(space of Retzius) gives extraperitoneal access. Infant bladder is intra-abdominal.
- Detrusor: inner longitudinal, middle circular, outer longitudinal. Bladder-neck circular fibres = **internal urethral sphincter**, densely **α1-adrenergic** → continence and **antegrade ejaculation**; damage contributes to post-prostatectomy incontinence.
- Body: dense parasympathetic cholinergic supply; non-adrenergic non-cholinergic (NANC) fibres potential drug targets → *Ch 103*.
- Arterial pedicles lie posterolaterally; veins form a plexus in the pedicles draining to the internal iliac vein. Lymph starts in the lamina propria → mainly **external iliac**, also internal iliac and obturator nodes, bilaterally.

### Ureterovesical junction

- Ureter in **Waldeyer's sheath** runs **1.5–2 cm** obliquely; narrow → stone impaction.
- **Flap valve**: detrusor backing compresses the intramural ureter during filling. Weak backing → **vesicoureteral reflux (VUR)**; weak hiatus → **Hutch diverticulum**.
- Superficial trigone (ureteric muscle) anchors at the verumontanum; deep trigone (Waldeyer's sheath + detrusor) at the bladder neck, anchoring the ureters.
- Pelvic ureter: narrows at the iliac crossing; runs laterally along the sidewall, beneath the umbilical artery, then medially **behind the vas** to the bladder. Extrinsic compression: gravid uterus, enlarged iliac nodes. Blood supply enters **laterally** → preserve lateral attachments; circumferential stripping → stricture.

## 8 · ENDOSCOPIC ANATOMY

**ALGORITHM**
Flexible cystoscopy (local) → bladder, orifices, trigone → retroflex at neck (intravesical prostatic protrusion, hidden tumours) → verumontanum with ejaculatory ducts → external sphincter coapts just distal → bulbar urethra

**KEY POINT** Verumontanum = sentinel at transurethral resection of the prostate (TURP): resection distal to it injures the external sphincter → incontinence.

## 9 · RADIOGRAPHIC ANATOMY

- **Phlebolith**: normal venous calcification mimicking a distal ureteric or bladder stone.
- **Cystogram**: filling bladder spreads laterally; absent lateral distension → extraperitoneal pelvic haematoma (trauma) or lymphocele after radical prostatectomy.
- **Voiding cystourethrography (VCUG)** best shows the posterior urethra (both sphincters relaxed). **Retrograde urethrography (RUG)**, oblique: external sphincter may resist — steady pressure fills it.
- Membranous urethra (crossing the urogenital diaphragm, just proximal to the cone of the bulb) = **shortest, narrowest** segment — not a stricture. Verumontanum = oval filling defect. Opacified Littré glands/Cowper's ducts are normal.
- †Pelvic fracture urethral injury typically occurs at the bulbomembranous junction.
- **Computed tomography (CT)**: high spatial, low contrast resolution → needs contrast except for **stones**; poor for primary tumours. Urine from **extraperitoneal rupture** collects in retropubic and perivesical spaces; retrovesical pouch collects fluid and abscess.
- Malignant node: rounded, irregular, heterogeneous; size most specific at ≥**1 cm** short axis.
- **CT urogram**: excretory images at **7–10 min**; iliac-crossing narrowing and peristaltic gaps are normal.

- **Magnetic resonance imaging (MRI)**: best soft-tissue contrast. T1 — bone, gadolinium enhancement; T2 — organ anatomy, fluid, prostate, seminal vesicles, pelvic floor. Distended bladder wall >**3 mm** = thickening. External sphincter = T2-dark doughnut below the apex.

## 10 · MCQ ONE-LINERS

1. Sacrospinous ligament divides the sciatic foramen into greater and lesser.
2. Iliac crest corresponds to L4; umbilicus T10.
3. Below arcuate line, posterior rectus sheath is absent.
4. Inguinal canal: spermatic cord and ilioinguinal nerve.
5. Indirect hernia lies lateral to inferior epigastric vessels.
6. Medial umbilical ligament = obliterated umbilical artery.
7. Butterfly haematoma: anterior urethral injury confined by Colles fascia.
8. Puborectalis: key to faecal continence.
9. Belt approach passes under the external anal sphincter.
0. Internal pudendal artery runs in Alcock's canal.
1. Deep circumflex iliac arises from external iliac.
2. Superior vesical arises from patent umbilical artery.
13. Inferior vesical artery supplies the neurovascular bundle.
14. Node of Cloquet: distal limit of PLND.
15. Genitofemoral nerve: lateral limit of PLND.
16. Obturator nerve injury → weak thigh adduction.
17. Femoral nerve injury: psoas hitch, retractor blade.
18. NVB at 5 and 7 o'clock, outside Denonvilliers fascia.
19. Bladder neck α1 closure → antegrade ejaculation.
20. Intramural ureter: 1.5–2 cm.
21. Posterior urethra best imaged by VCUG.
22. Membranous urethra narrowing is normal.
23. CT urogram excretory phase: 7–10 minutes.
24. Phlebolith mimics a distal ureteric stone.
25. Distended bladder wall on MRI >3 mm = thickening.
26. Commonest rectal injury site at prostatectomy: distal rectum near apex.
27. Arcus tendineus fascia pelvis: puboprostatic ligament → ischial spine.

# 103 Physiology and Pharmacology of the Bladder and Urethra

*Reading time ≈ 51 min (slow pace)*

## 1 · OVERVIEW

- The lower urinary tract (LUT: bladder + urethra) has two defining functions: **storage** (continence) and **emptying** (micturition).
- Control integrates the central nervous system (CNS), peripheral nerves, detrusor smooth muscle (DSM), urothelium, urethral muscle, pelvic floor and external urethral sphincter (EUS).
- Lower urinary tract dysfunction (LUTD) includes urinary incontinence (UI), lower urinary tract symptoms (LUTS), overactive bladder (OAB), bladder pain syndrome/interstitial cystitis (BPS/IC), neurogenic bladder and underactive bladder (UAB).

> **KEY POINT** **Why single-organ therapy fails:** bladder and urethra act reciprocally, so boosting contractility in detrusor underactivity fails unless the urethra also relaxes. Afferent (sensory) modulation acts on both at once — reduce it for overactivity, augment it for underactivity — which makes it the most promising therapeutic lever.

## 2 · FUNCTIONAL ANATOMY

### Bladder

- **Body** lies above the ureteric orifices; **base** = trigone + bladder neck. The two regions differ in neuromorphology and neuropharmacology but each is internally homogeneous.
- **Male bladder neck**: complete smooth muscle ring with dense sympathetic noradrenergic supply; it closes during ejaculation → **antegrade ejaculation**. Female: no such collar, sparse adrenergic supply.
- **Lamina propria** = proposed "functional centre": unmyelinated nerve plexus with TRPV1 and P2X3 receptors, interstitial cells (myofibroblasts) and a subepithelial capillary plexus (the vessels seen at cystoscopy).
- **Blood flow** falls with distension; intramural tension is the main determinant. In low compliance, pressure rises more and flow falls more. Ischaemia rapidly reduces contractility; ischaemia–reperfusion damages intramural neurons → patchy denervation, as seen in detrusor overactivity (DO).

### Stroma, collagen and elastin

- Wall ≈ **50% collagen, 2% elastin** besides muscle. Collagen mostly outside muscle bundles; types I, III, IV predominate.
- **Poorly compliant bladders**: ↑ connective tissue : smooth muscle ratio and ↑ **type III : type I collagen**; collagen fibres lose the ability to change tortuosity → ↓ capacity.

### Urethra and sphincters

- Bladder outlet = urethra + pelvic floor. Urethra has smooth muscle (lissosphincter) and an **intramural striated rhabdosphincter (EUS)**, separate from periurethral pelvic floor muscle.
- **Male urethra**: preprostatic (bladder neck) → prostatic → membranous (through pelvic floor, incl. EUS) → bulbous and penile. Male EUS: crescent ventral to prostate proximal to verumontanum → horseshoe distally → crescent at bulbar urethra.
- **Female urethra**: in distal third of anterior vaginal wall; EUS a horseshoe over the ventral urethra; subepithelial vascular plexus forms a **"urethral seal"**.
- Levator ani does not encircle the ventral urethra — its direct continence role is questioned.

| Feature | Slow-twitch | Fast-twitch |
|---|---|---|
| Role | Sustained resting tone | Rapid extra squeeze on coughing |
| Fatigue / metabolism | Slow; oxidative | Rapid; anaerobic |
| Where | Intramural rhabdosphincter (predominant) — gives **~50% of static urethral resistance** | Periurethral pelvic floor (mixed) |
| Rhabdosphincter mix | Male **65% slow / 35% fast**; female **87% slow / 13% fast** | |

- Pelvic floor exercise/electrostimulation may help stress urinary incontinence (SUI) by converting fast to slow fibres (speculative).

## 3 · UROTHELIUM: BARRIER AND SENSOR

### Barrier

- ~7 layers: basal, intermediate, apical **umbrella cells** (largest, hexagonal, sometimes multinucleate).
- **Uroplakins** (unique to umbrella cells) form plaques of **asymmetrical unit membrane (AUM)** — outer leaflet thicker; joined by hinge regions. They provide the barrier and are the binding site of **type 1-fimbriated *E. coli***.
- **Tight junctions** (occludin, claudins, zona occludens-1) block paracellular flow. Stretch lowers transepithelial resistance tenfold (ionic paracellular transport) without changing small-molecule permeability.
- Loss of umbrella cells (cystitis, infective/radiation) → solute leak into lamina propria → symptoms worsened by distension; hyperosmolality excites C-fibre afferents.
- **Glycosaminoglycan (GAG) layer**: controversial. The "antiadherence factor" proved to be a glycoprotein (mucin MUC-1); chondroitin sulfate is the main human GAG. GAG may aid antiadherence but is **not proven** to be the primary impermeability barrier.

### Sensor–transducer function

Urothelial cells behave like sensory neurons: they express receptors for bradykinin, neurotrophins (TrkA, p75), purines (P2X, P2Y), noradrenaline (α, β), acetylcholine (ACh; nicotinic, muscarinic), ENaC and transient receptor potential (TRP) channels (TRPV1, TRPV2, TRPV4, TRPM8). On stretch or chemical stimulation they release mediators onto adjacent afferents, directly or via suburothelial myofibroblasts.

| Mediator | Released by / when | Effect |
|---|---|---|
| **ATP** | Stretch; ↑ with injury, inflammation, ageing | Excites suburothelial **P2X3** (and P2X2/3) afferents → fullness, pain; P2X3-null mice are hyporeflexic |
| **Nitric oxide (NO)** | Especially in inflammation | Little effect on detrusor; inhibits afferent $Ca^{2+}$ channels → net **afferent inhibition** |
| **ACh** (non-neuronal) | Basal; ↑ by stretch and ageing | Modulates afferents and DSM; muscarinic receptors on urothelium → antimuscarinics may act partly here |
| Prostaglandins (PGs) | Urothelium = main source; ↑ in inflammation | Detrusor regulation, cytoprotection |
| Inhibitory factor (unknown) | Basal | ↓ muscarinic contraction; not NO or PG |

- Normal afferent thresholds reflect an **ATP (excitatory) : NO (inhibitory) balance**; ATP ↑ in DO and BPS/IC.
- **Suburothelial myofibroblasts**: vimentin+, α-smooth muscle actin+, desmin−; coupled by **connexin 43 (Cx43)** gap junctions; close to C-fibre endings; respond to ATP (P2Y6) and ACh

(M2/M3). In idiopathic DO (IDO), their M2/M3 expression correlates with urgency. NO donors damp them via cyclic guanosine monophosphate (cGMP).

> **KEY POINT** A damaged urothelium produces symptoms twice over — by **solute leak** into the lamina propria and by **altered signalling** (more ATP, less NO) to afferents.

## 4 · DETRUSOR SMOOTH MUSCLE PHYSIOLOGY

### Structure

- Fascicles in random directions coupled as a functional syncytium (unlike the discrete layers of ureter and gut). Cells spindle-shaped, single nucleus, no striations, a few hundred µm × 5–6 µm.
- No sarcomeres: thick (myosin II), thin (actin + tropomyosin + caldesmon) and intermediate (desmin, vimentin) filaments anchor to **dense bodies**.
- Motor supply: postganglionic parasympathetic varicosities release ACh and ATP; not every cell is contacted — **gap junctions** spread excitation.

| Property | Skeletal muscle | Smooth muscle |
|---|---|---|
| Innervation | Somatic | Autonomic + hormonal, paracrine |
| Activation | $Ca^{2+}$–troponin removes tropomyosin inhibition; rapid | $Ca^{2+}$–calmodulin → **myosin phosphorylation**; slow; "latch state" |
| Contraction / tone | Phasic; tone neural | Mostly tonic; intrinsic + extrinsic tone |

### Excitation–contraction coupling

**ALGORITHM**
ACh → **M3** (Gq) → ↑ $[Ca^{2+}]i$ (L-type $Ca^{2+}$ entry + phospholipase C → IP3 → SR release) → $Ca^{2+}$–calmodulin → myosin light chain kinase (MLCK) → phosphorylates MLC20 → actin–myosin cross-bridge cycling → contraction → myosin light chain phosphatase (MLCP) dephosphorylates as $Ca^{2+}$ falls → relaxation

- Tone = $Ca^{2+}$ balance + **MLCK : MLCP balance**. **Calcium sensitisation**: RhoA-GTP → Rho kinase (ROK) and protein kinase C (via CPI-17) inhibit MLCP → force rises with no rise in $Ca^{2+}$. Muscarinic activation recruits ROK.
- **L-type voltage-dependent $Ca^{2+}$ channels (VDCC)** are the main contraction trigger (blocked by nifedipine, diltiazem, verapamil); in mouse DSM nerve-evoked contraction depends on L-type entry alone. T-type: finer control.
- Resting potential −50 to −60 mV. Action potential upstroke = $Ca^{2+}$ inward current; repolarisation = $K^+$ outward current via SK, **BK** (large-conductance $Ca^{2+}$-activated $K^+$) and Kv channels. **BK is the key brake**: $Ca^{2+}$ sparks from ryanodine receptors open BK → hyperpolarisation. BK knockout → OAB phenotype; BK ↓ in neurogenic DO (NDO).

| G protein | Receptors | Effect |
|---|---|---|
| Gq | M1, M3, M5 ("odd"); α1; angiotensin II; H1; 5-HT2 | IP3 + DAG → $Ca^{2+}$ release, PKC → contraction |
| Gi/o | M2, M4 ("even") | ↓ adenylyl cyclase → ↓ cAMP |
| Gs | β-adrenoceptors (β3) | ↑ cAMP → protein kinase A → relaxation |

### Purinergic contraction and DHIC

- Atropine-resistant (purinergic, P2X1) contraction is **minimal in normal human bladder** but appears in obstructed, OAB and ageing bladders.
- Purinergic activation suppresses muscarinic coupling → explains **detrusor hyperactivity with impaired contractility (DHIC)**: ↑ afferent drive (urgency) + ↓ muscarinic contraction (incomplete emptying).

### Gap junctions and myogenic activity

- Cx43 (and Cx45) gap junctions couple cells within bundles, but the detrusor is deliberately **poorly coupled** overall — high tissue impedance, no fused tetanic contractions — which protects against synchronous activation during filling.
- ↑ Cx43 coupling (BOO, NDO, urgency; induced by cytokines and nerve growth factor, NGF) → coordinated whole-bladder myogenic contraction = DO.
- **Interstitial cells**: intradetrusor c-KIT+ cells (modulate $Ca^{2+}$ transients; imatinib ↓ spontaneous contractions; ↑ in OAB/BOO) and inhibitory PDGFRα+ cells (P2Y1-mediated relaxation).

> **KEY POINT** **Motor–sensory coupling:** afferents lie beside myocytes; spontaneous micromotions generate afferent bursts perceived as urgency. Antimuscarinics and mirabegron reduce non-voiding contractions and hence sensory input — efferent and afferent functions are inseparable.

## 5 · RECEPTORS AND DRUG TARGETS

| Receptor | Location | Function | Drug target |
|---|---|---|---|
| **M3** (Gq) | Detrusor; urothelium; salivary glands | **Mediates contraction** (knockout proves it) | Antimuscarinics (OAB) |
| **M2** (Gi) | Detrusor (**numerically predominant**); urothelium | Amplifies M3: opposes β-cAMP relaxation, inactivates $K^+$ channels, opens cation channels; shift M3 → M2 in neurogenic/obstructed bladder | Non-selective antimuscarinics |
| **β3** (Gs) | Detrusor (**most expressed adrenoceptor mRNA**); sacral cord; afferents | Relaxation via cAMP during storage; ↑ compliance, ↓ non-voiding contractions | Mirabegron, †vibegron (OAB) |
| **α1A** | Urethra, prostate (dominant) | Urethral contraction / closure; maintains resting female urethral tone | α-blockers ↑ flow in benign prostatic hyperplasia (BPH); α-agonists ↑ closure |
| **α1D** | Bladder (normal human predominant α); spinal cord | Storage symptoms of obstruction | α1D-active blockers (tamsulosin) improve storage LUTS |
| α2 | Prejunctional urethra; vesical ganglia | ↓ noradrenaline release, ↓ ganglionic transmission | — |
| **P2X3** (P2X2/3) | Suburothelial unmyelinated afferents | Senses urothelial ATP → fullness, pain; ↑ in BPS/IC | P2X3 antagonist AF-219 (BPS/IC proof-of-concept) |
| P2X1 | Detrusor, vascular muscle | Purinergic contraction; ↑ in BOO, ageing | Potential for DHIC |
| **TRPV1** | C-fibre nociceptors; urothelium | Heat >43 °C, protons, capsaicin; inflammatory hypersensitivity; ↑ in NDO | Capsaicin, resiniferatoxin (RTX) desensitise |
| TRPM8 | Afferents | Cold → bladder cooling reflex | Experimental |
| NK1 (substance P) | Spinal cord, vessels | Afferent transmission, neurogenic inflammation | Aprepitant ↓ urgency in OAB (trial) |
| Nicotinic | EUS (pudendal) | Striated sphincter contraction | — |

### Antimuscarinics

- **Tissue selectivity matters more than subtype selectivity** — salivary glands also use M3, so M3-selective darifenacin (~30-fold M3 > M2) still causes dry mouth. Tolterodine and solifenacin are relatively bladder-selective.

- **Oxybutynin**: non-selective; its metabolite **desethyloxybutynin** causes most dry mouth → controlled-release oxybutynin keeps efficacy with fewer side effects.
- **Cognition**: lipophilic, blood–brain barrier-crossing agents block CNS M1 → confusion, delirium, cognitive decline; additive anticholinergic burden. Caution in frail elderly/dementia; effects reverse within weeks of withdrawal.

### Other storage drugs (Table 103.4)

- Bladder-acting: antimuscarinics, direct relaxants, $Ca^{2+}$ antagonists, $K^+$ channel openers, β-agonists, tricyclics (imipramine: anticholinergic + direct relaxant + noradrenaline reuptake inhibition), afferent inhibitors (capsaicin, RTX).
- Outlet-acting: **α-agonists** (ephedrine, midodrine) ↑ urethral closure; **oestrogen** ↑ mucosal seal, blood flow and urethral adrenergic response — but alone disappointing for SUI/urgency urinary incontinence (UUI); systemic oestrogen may worsen UI.
- **Duloxetine** (serotonin–noradrenaline reuptake inhibitor): ↑ Onuf's nucleus output → ↑ striated sphincter activity; beats placebo in mild–moderate SUI; approved in Europe, not USA.

## 6 · PERIPHERAL INNERVATION

| Feature | Parasympathetic | Sympathetic | Somatic |
|---|---|---|---|
| Origin | **S2–S4** sacral parasympathetic nucleus (intermediate grey) | Thoracolumbar intermediolateral cell column (†**T10–L2**) | **Onuf's nucleus**, S2–S4 ventral horn (lateral border) |
| Nerve | Pelvic | Hypogastric | Pudendal |
| Bladder body | Contracts (ACh → M3; ATP → P2X1) | Relaxes (noradrenaline → β3) | — |
| Bladder neck / urethra | Relaxes urethra (**NO**) | Contracts (α1) | Contracts EUS (ACh → nicotinic) |
| Ganglia | Pelvic plexus + **intramural** | Chain → inferior mesenteric → pelvic | None |
| Net role | **Voiding** | **Storage** | Storage (voluntary closure) |

- All three nerves are mixed afferent + efferent.
- Postganglionic parasympathetic neurons lie partly in the bladder wall, so cauda equina or pelvic plexus injury **decentralises rather than fully denervates** the bladder.

### Afferents

- Pelvic and pudendal afferents → sacral dorsal root ganglia (DRG); hypogastric → rostral lumbar DRG. Lamina propria plexus densest at bladder neck/proximal urethra, absent at the dome.
- Filling gives episodic sensory bursts of rising intensity; the last is perceived as urgency. Afferent firing during contraction reinforces voiding.

| Feature | Aδ (thin myelinated) | C (unmyelinated) |
|---|---|---|
| Location | Smooth muscle | Mucosa (± muscle) |
| Normal role | Low-threshold **tension receptors** — fullness; drive normal reflex | Majority; many "**silent**" to normal distension; nociception |
| Reflex | Spinobulbospinal (via pons) | New **spinal reflex** after spinal cord injury (SCI), inflammation, BOO, ageing |
| Capsaicin / RTX | Resistant | Sensitive (blocked) |

- **Sensitisation** (leftward shift of stimulus–response curve) by ATP, NGF (→ TrkA), PGs, bradykinin, high $K^+$, low pH, high osmolality; persistent injury → central sensitisation.

- **Cross-organ sensitisation** (shared DRG/spinal convergence) explains BPS/IC–irritable bowel overlap.
- μ-opioids ↓ bladder sensation and ↑ sphincter tone → postoperative retention.

## 7 · CENTRAL CONTROL

### Spinal transmitters

- **Glutamate**: main excitatory transmitter of the reflex.
- **GABA, glycine**: inhibit micturition (mediate rectovesical inhibition).
- **Serotonin**: inhibits parasympathetic drive, excites sphincter. **Dopamine** tonically inhibits the PMC — lost in Parkinson disease → overactivity.

### Brainstem switch: PAG and PMC

- **Pontine micturition centre (PMC) = Barrington's nucleus** = "M region" (dorsal pontine tegmentum). Stimulation → bladder contraction + sphincter relaxation; lesions (stroke, MS, tumours) → **retention**.
- PMC does **not** project to Onuf's nucleus; it relaxes the sphincter **indirectly** via GABA/glycine inhibitory interneurons in the dorsal grey commissure — the command centre for emptying.
- **L-region** (ventrolateral pons) = pontine continence/storage centre; projects to Onuf's nucleus.
- **Periaqueductal grey (PAG)** receives ascending bladder afferents and relays to the PMC; together PAG + PMC form a binary "on–off" switch. Without higher control, the bladder empties involuntarily at a critical volume (infant).

### Forebrain

- **Storage sequence**: afferents → PAG (switch not tripped) → thalamus → **insula** (desire to void) → lateral/medial **prefrontal cortex** (decision, social appropriateness) → medial frontal → brainstem pathway **tonically suppresses the reflex** = normal continence mechanism.
- Frontal white-matter (medial prefrontal–PAG) lesions → **incontinence**; dorsal anterior cingulate gyrus and supplementary motor area over-respond in urge incontinence (urgency = emergency "backup" mechanism).

> **KEY POINT** **Lesion localisation logic:** forebrain lesion → loss of tonic inhibition → DO with preserved coordination; pontine lesion → retention; suprasacral cord lesion → loss of PMC coordination → DO + **detrusor–sphincter dyssynergia (DSD)**; sacral/cauda lesion → decentralised, acontractile bladder → *Ch 104*.

## 8 · STORAGE AND EMPTYING PHASES

| Phase | Afferent | Efferent | Central pathway |
|---|---|---|---|
| **Storage** | Low-level pelvic afferent activity | EUS contraction (somatic); internal sphincter contraction, detrusor and ganglionic inhibition (sympathetic); parasympathetic silent | **Spinal** reflexes |
| **Micturition** | High-level pelvic afferent activity | EUS inhibited; sympathetic inhibited; parasympathetic to bladder (contract) and urethra (NO relax) | **Spinobulbospinal** reflex via PAG–PMC |

### Storage

- Low constant pressure depends on: intrinsic muscle/stroma properties, quiescent parasympathetic outflow, urothelial apical membrane changes, and the **sympathetic storage reflex** (lumbosacral; closes outlet, inhibits detrusor). This reflex is weak in humans — bilateral retroperitoneal lymph node dissection does not alter storage.
- **Guarding reflex**: rising bladder afferent activity recruits pudendal motoneurons → EUS electromyography (EMG) increases with filling. Also triggered by sudden abdominal pressure

(cough) — urethral pressure rises **before** pressure transmission, proving an active neural component. Its suppression during voiding is supraspinal → lost after SCI → **DSD**.

- **Sphincter-to-bladder reflex**: pudendal afferents (sphincter, perineum, anal canal, vagina, penis) inhibit bladder preganglionic neurons and interneurons — basis of "holding on" by pelvic floor contraction and of neuromodulation.

### Emptying

**ALGORITHM**
Tension-receptor (Aδ) firing exceeds threshold → PAG/PMC switch flips (if cortex permits) → sympathetic and pudendal outflow off → **urethral sphincter relaxes first** (seconds earlier) + urethral smooth muscle relaxes (NO) → detrusor contracts (parasympathetic) → flow → urethral flow afferents reinforce contraction until empty

- **Urethra-to-bladder reflexes** (Barrington): flow facilitates bladder contraction — a somatic pudendal component via the PMC and a visceral pelvic spinal component. Explains a link between stress incontinence (urine entering the proximal urethra) and detrusor instability.
- After SCI: spinal shock (retention for weeks) → a **C-fibre spinal micturition reflex** emerges via synaptic reorganisation, driven by NGF → NDO with DSD; reciprocity is lost. Ice-water test = research tool only.

### Voiding mechanics

- **Pdet = Pves − Pabd**. Detrusor power ≈ Pdet × flow (Q) is roughly constant, so voiding **Pdet reports outlet resistance, not contractile strength**.
- SUI women void at Pdet at Qmax ≈ **19 $cmH_2O$**; healthy men ≈ **60–70 $cmH_2O$**. Low voiding pressure in a woman ≠ impaired contractility; male obstruction nomograms must not be applied to women.

## 9 · URETHRAL CLOSURE MECHANISMS

- **Contributors to urethral pressure**: smooth muscle, striated muscle and lamina propria/vascular plexus. Nicotinic blockade rarely lowers tone >40%; α-blockade lowers urethral pressure by about **one third** → smooth muscle (α1A) tone matters; little cholinergic contribution.
- EUS pressure rises uniformly like an **iris** despite its posteriorly open horseshoe; noradrenaline augments it.
- **Mucosal seal**: vascular filling of lamina propria; oestrogen ↑ urethral blood flow. Impaired arterial supply ↓ urethral pressure (early vascular, later hypoxic muscle effect).
- **Passive pressure transmission** to the proximal urethra during raised abdominal pressure.
- **Hammock hypothesis (DeLancey)**: abdominal pressure compresses the urethra against a posterior layer kept firm by pelvic muscle/fascial support (pubourethral and vaginal–fascial attachments).
- **Active** guarding reflex: fast-twitch fibres add a reflex squeeze on coughing.

> **KEY POINT** Continence = **active muscle tone + passive anatomical coaptation**. The integral theory led to the mid-urethral sling (MUS), the surgical standard for SUI, yet DeLancey's data suggest intrinsic sphincter function matters more than support → *Ch 109*.

## 10 · COMPLIANCE

- **Compliance C = ΔV / ΔP**. Physiological filling keeps pressure **<10 $cmH_2O$**; women with SUI filled at 50 mL/min rose only a few $cmH_2O$ to capacity.
- Determinants: (1) passive collagen, elastin, muscle viscoelasticity; (2) active muscle tone; (3) geometry. During filling the lamina propria thins, bundles realign side-to-side and coiled **type III collagen straightens** parallel to the lumen.

- **Low compliance** (steep curve): fast filling, ↑ collagen/↓ elastin, smooth muscle hyperactivity, or combinations.
- Compliance is **fill-rate dependent**: pressure falls when urodynamic filling slows or stops — so slow the pump before diagnosing low compliance.
- Active component: acute spinal transection does not alter compliance, but pelvic nerve transection reduces it → an afferent–CNS loop inhibits spontaneous contractility during filling.
- † A detrusor leak point pressure ≥ **40 $cmH_2O$** in low-compliance neurogenic bladders predicts upper tract damage — compliance is the main renal-risk parameter → *Ch 104*.

## 11 · PATHOPHYSIOLOGY LINKS

### Idiopathic OAB / DO

- DO is found in only a minority (11% in one OAB cohort) — symptoms reflect dysregulated sensation more than contraction → "afferent neurourology".
- Mechanisms: ↑ urothelial ATP/TRPV1, ↑ Cx43 coupling, ↓ BK, NGF-driven plasticity.

### Bladder outlet obstruction (BOO)

- Detrusor hypertrophy (no mitoses), partial denervation → **muscarinic supersensitivity**, ↑ P2X1 and Cx43 → coordinated myogenic contraction (myogenic theory, Brading).
- Neural: NGF from strained DSM enlarges afferent/efferent neurons and builds new spinal circuits; NGF stays ↑ when frequency persists after relief — explaining **persistent storage symptoms after prostate surgery**.

### BPS/IC

- Afferent hyperexcitability: ↑ NGF, ATP/P2X3, TRPV1, mast cell activation. Single-target trials (anti-NGF tanezumab, intravesical RTX) failed — likely patient heterogeneity → *Ch 113*.

### Underactive bladder

- UAB = symptom complex (weak stream, hesitancy, straining); detrusor underactivity = urodynamic finding. Afferent loss may cause underactivity, not just efferent failure.
- Post-void residual (PVR) ≥ **300 mL** warrants further evaluation (also if bothersome LUTS, hydronephrosis, renal insufficiency, culture-proven UTI); PVR <300 mL does not prove normal contractility.

## 12 · AGEING

- Ageing = **hypofunction**: detrusor contractility, bladder sensation and urethral closure pressure all decline. Capacity does not fall with age per se — smaller capacity reflects coexisting DO.
- Structure: ↓ smooth muscle relative to collagen; pelvic ischaemia → LUTS, DO.
- Nerves: ↓ cholinergic nerves, ↓ afferent neuropeptides, ↓ brain response to filling.
- Receptors: cholinergic response unchanged (despite ↓ muscarinic density); **α-contraction ↑ (α1D) and β-relaxation ↓** → noradrenaline contracts the aged bladder but relaxes the young one.
- Aged CNS is prone to overactivity despite ↓ baseline function → urgency with poor emptying (DHIC).
- Antimuscarinics: anticholinergic burden linked with dementia risk (causality unclear) → †prefer β3 agonists or low-CNS-penetrant agents in the elderly.

## 13 · NEUROMODULATION AND BOTULINUM TOXIN — MECHANISMS

- **Sacral neuromodulation (SNM)**, pudendal and posterior tibial nerve stimulation (PTNS) treat refractory OAB **and** non-obstructive urinary retention (NOUR). Target = **afferent axons**.
- **Paradox resolved**: in OAB, somatic afferent input inhibits bladder preganglionic neurons and interneuronal transfer to the PMC; in retention, it switches off the overactive **guarding reflex**, releasing the bladder.

- Pudendal stimulation carries S2–S4 afferents and avoids sciatic-territory discomfort of S3 stimulation.
- **OnabotulinumtoxinA (OBTX)**: heavy chain binds nerve terminal → internalisation → light chain (zinc protease) cleaves **SNAP-25** → no vesicle fusion → ↓ ACh release. Also afferent: ↓ TRPV1/P2X3 in C fibres, ↓ SP, CGRP and urothelial ATP.

## 14 · LANDMARK TRIALS

| Trial | Design | Result | Take-home |
|---|---|---|---|
| ABC | RCT: intradetrusor OBTX vs oral antimuscarinic, idiopathic OAB with UUI | Equal ↓ in daily UUI; OBTX more complete dryness | Antimuscarinic: dry mouth; OBTX: catheterisation, UTI |
| ROSETTA | RCT: OBTX 200 U vs SNM, women with refractory non-neurogenic OAB, 2 years | Comparable efficacy; OBTX more often dry | OBTX more UTIs (note 200 U dose) |

## 15 · MCQ ONE-LINERS

1. Muscarinic subtype most numerous in detrusor: **M2**; subtype mediating contraction: **M3**.
2. Most highly expressed adrenoceptor in human detrusor: **β3**.
3. Dominant α-subtype in urethra and prostate: **α1A**; in human bladder: **α1D**.
4. Main $Ca^{2+}$ source triggering detrusor contraction: **L-type VDCC**.
5. Calcium sensitisation: **Rho kinase** inhibits MLCP.
6. Key $K^+$ channel braking detrusor excitability: **BK** (large-conductance $Ca^{2+}$-activated).
7. Uroplakins bind **type 1 fimbriae** of uropathogenic *E. coli*.
8. Urothelial transmitter exciting afferents via P2X3: **ATP**; inhibiting them: **NO**.
9. Parasympathetic transmitter relaxing urethra during voiding: **NO**.
0. Somatic EUS motoneurons: **Onuf's nucleus** (S2–S4 ventral horn).
1. PMC = **Barrington's nucleus** (dorsal pontine tegmentum); lesion → retention.
2. Pontine storage centre projecting to Onuf's nucleus: **L-region**.
13. Normal micturition reflex: **spinobulbospinal**, Aδ afferents.
14. Reflex emerging after SCI: **C-fibre spinal**, capsaicin/RTX-sensitive.
15. Dopamine tonically **inhibits** PMC — lost in Parkinson disease.
16. Loss of supraspinal suppression of guarding reflex → **DSD**.
17. Female rhabdosphincter: **87%** slow-twitch fibres.
18. Collagen type ↑ in poorly compliant bladder: **type III**.
19. α-blockade lowers urethral pressure by about **one third**.
20. Oxybutynin dry mouth mainly from metabolite **desethyloxybutynin**.
21. OBTX cleaves **SNAP-25**.
22. Duloxetine acts on **Onuf's nucleus** to ↑ sphincter tone.
23. Aged bladder: α-contraction ↑, β-relaxation ↓.
24. Neurotrophin central to DO plasticity: **NGF**.
25. RTX desensitises TRPV1 **without prior excitation**, unlike capsaicin.

# 104 Pathophysiology and Classification of Lower Urinary Tract Dysfunction

*Reading time ≈ 16 min (slow pace)*

## 1 · NORMAL FUNCTION: TWO PHASES

The lower urinary tract (LUT) stores at low pressure with continence and empties completely, voluntarily, at low pressure. Phases switch on–off: storage reflexes on/voiding off ⇄ the reverse.

| Storage requires | Voiding requires |
|---|---|
| Low detrusor pressure (normal compliance), appropriate sensation | Detrusor contraction of adequate magnitude and duration |
| Outlet closed at rest and on straining | Fall in smooth and striated sphincter resistance |
| No detrusor overactivity (DO) | No anatomic obstruction |

- **Storage is mostly passive**: compliance (Δvolume/Δpressure) from elastic/viscoelastic stroma. ↑ collagen (inflammation, bladder outlet obstruction [BOO], decentralisation) → low compliance, then **drug-unresponsive**.
- Storage reflexes: α1 → bladder neck tone; β3 → detrusor relaxation; spinal **guarding reflex** raises urethral pressure.
- **Voiding**: pelvic-nerve parasympathetic outflow, coordinated by the **pons**; outlet relaxes first (nitric oxide), then detrusor contracts.
- **Stress continence**: coughing does not open the outlet because no coordinated contraction is triggered. Protection = at least equal pressure transmission to proximal urethra (**Enhorning**) plus active reflex closure (**Tanagho**). Its failure accompanies all effort-related urinary incontinence (UI).
- Urethral closure also needs a **mucosal seal** — scarred or atrophic mucosa seals poorly. **Klevmark**: the cystometric pressure rise is an artefact of fast filling.

## 2 · PATHOPHYSIOLOGY

Excluding psychological causes, every failure is due to the **bladder, the outlet, or both** — the basis of the **Wein functional classification**. Failure is usually relative; combinations are common.

### Storage failure

- **Bladder overactivity** = involuntary contractions and/or low compliance: neurological disease, BOO, ageing, ↑ afferent input, idiopathic.
- **Overactive bladder (OAB)** (International Continence Society, ICS): urgency ± urgency urinary incontinence (UUI), usually with frequency and nocturia. UUI → assume DO; urodynamic studies (UDS) often miss it. Mechanisms: ↓ suprapontine inhibition, axonal damage, enhanced excitatory transmission, ↑ afferent input. Pelvic floor contraction reflexly inhibits the detrusor — basis of bladder training.
- **Hypersensitivity** without overactivity: classically interstitial cystitis/bladder pain syndrome (IC/BPS).
- **Low compliance**: sacral/infrasacral injury, fibrosis from long-standing BOO.
- **Outlet underactivity**: sphincter denervation, surgery/trauma, ageing, loss of female support.

| | Hypermobility (genuine stress urinary incontinence, SUI) | Intrinsic sphincter deficiency (ISD) |
|---|---|---|
| At rest | Outlet competent | Bladder neck poorly functional/open |
| Cause | Poor pelvic support | Prior surgery, scarring, neurological |
| Treatment | Correct hypermobility | Coaptation/compression |

**KEY POINT** UI can occur from ISD alone, **never from hypermobility alone**. **DeLancey's hammock**: a firm suburethral layer lets abdominal pressure compress the urethra. In half of continent women the continence point is the **mid-urethra**, not the bladder neck. Male sphincteric incontinence resembles ISD, not hypermobility.

**Emptying failure**

- **Bladder underactivity**: neurogenic (incl. pelvic surgery), myogenic (overdistension, drugs, infection), psychogenic; diabetes = neuromuscular + vascular.
- **BOO** (ICS): low flow and/or raised post-void residual (PVR) **with high detrusor pressure**. Anatomic — male: prostate, bladder neck contracture, stricture; female: prolapse, post-sling.
- **Functional**: striated sphincter dyssynergia (neurological, commonest); smooth sphincter dyssynergia (autonomic dysreflexia); primary bladder neck obstruction (young men); dysfunctional voiding (non-neurogenic).

## 3 · FUNCTIONAL CLASSIFICATION (WEIN) — 2 × 2 TABLE

| | Because of the bladder | Because of the outlet |
|---|---|---|
| **Failure to store** | DO; low compliance; hypersensitivity; underactivity with overflow | Hypermobility; ISD. (Plus **fistula**) |
| Treatment | Bladder training; antimuscarinics, β3-agonists, onabotulinumtoxinA; neuromodulation; augmentation | Pelvic floor training; α-agonists, duloxetine; bulking; slings; artificial urinary sphincter |
| **Failure to empty** | Underactivity: neurogenic, myogenic, psychogenic | Anatomic obstruction; sphincter dyssynergia; bladder neck dysfunction |
| Treatment | Valsalva/Credé; parasympathomimetics; electrical stimulation | α-blockers, 5α-reductase inhibitors; prostatectomy, incision; sphincter onabotulinumtoxinA, sphincterotomy |
| **Circumvent** | Pads/collecting devices, antidiuretics, catheterisation, diversion | |

**KEY POINT**

- Bladder underactivity appears under **both** storage (overflow) and emptying. MCQ
- α-agonist tightens outlet (storage); α-blocker relaxes it (emptying).
- OnabotulinumtoxinA: intradetrusor → storage; sphincteric → emptying.
- Naming the failure names the treatment family — no exact diagnosis needed.

**Combined deficits** (T10 paraplegia: DO + sphincter dyssynergia): treat one as primary — circumvent emptying with external collection, or **abolish DO** and empty by clean intermittent catheterisation (CIC). Other combinations: BOO with DO; DO with impaired contractility. Chronic obstruction causes hypertrophy, collagen, diverticula and DO — intervene promptly.

## 4 · URODYNAMICS BY PHASE AND SITE

| Phase | Bladder | Outlet |
|---|---|---|
| Storage | Filling cystometry; detrusor leak point pressure (DLPP) | Urethral pressure profile; Valsalva leak point pressure (VLPP) |
| Voiding | Voiding cystometry (Pdet) | Fluoroscopy; sphincter electromyography |
| Both | Flowmetry and PVR — **cannot localise** the fault | |

## 5 · OTHER CLASSIFICATIONS

- **Symptoms (ICS)** — patient report, directs testing. Storage: frequency, nocturia, urgency, UI. Voiding: slow stream, intermittency, hesitancy, straining, **terminal dribble**. Post-micturition: incomplete emptying, **post-micturition dribble**.

- **Signs (ICS)**: bladder diary, examination (SUI, prolapse may mask UI), pad test.

**ICS urodynamic classification (most precise)**

| Domain | Storage | Voiding |
|---|---|---|
| Detrusor | Normal; DO (neurogenic/idiopathic) | Normal; underactive; acontractile |
| Sensation, capacity, compliance | Normal, high/increased, low/reduced, absent | — |
| Urethra | Normal or incompetent closure | Normal; BOO, dysfunctional voiding, detrusor–sphincter dyssynergia (DSD) |

- Cystometric capacity = permission to void given; maximum cystometric capacity = can no longer delay; maximum anaesthetic capacity = filled under anaesthesia.
- DSD = detrusor contraction with concurrent involuntary striated sphincter contraction; prefix "neurogenic" when a relevant neurological disorder exists.

Contrast: T10 paraplegia — neurogenic DO, absent sensation, DSD; stroke — neurogenic DO, normal sensation, **synergic** voiding.

† **Lapides**: sensory neurogenic, motor paralytic, uninhibited, reflex, autonomous bladder. **Bors–Comarr**: upper/lower motor neuron, complete/incomplete, balanced/unbalanced. MCQ

## 6 · NOCTURIA AND NEUROGENIC DYSFUNCTION

- **Polyuria >40 mL/kg/24 h**. **Nocturnal polyuria**: night volume ÷ 24-h volume **>20% young, >33% elderly**; from loss of nocturnal vasopressin rise or obstructive sleep apnoea.
- **IC/BPS**: urgency is driven by pain/pressure worsening with filling. Normally silent unmyelinated **C fibres** become hyperactive (central sensitisation); myelinated **Aδ fibres** normally signal distension. Bladder–bowel crosstalk explains OAB–irritable bowel overlap.
- **Neurogenic LUT dysfunction** (ICS) requires a clinically confirmed relevant neurological disorder.

| Lesion | Detrusor | Sphincter |
|---|---|---|
| Suprapontine | DO | Synergic |
| Suprasacral spinal | DO | DSD, raised PVR |
| Sacral | Acontractile | Impaired |

> **KEY POINT** DSD needs an intact sacral arc **disconnected from the pons** — hence suprasacral, never suprapontine.

## 7 · MCQ ONE-LINERS

1. Wein: failure to store or empty, due to bladder or outlet.
2. Storage failure outside bladder and outlet: fistula.
3. Micturition coordinating centre: pons.
4. Stress continence: equal pressure transmission to proximal urethra (Enhorning).
5. Collagen-driven low compliance is drug-unresponsive.
6. Flow and PVR cannot distinguish weak detrusor from obstruction.
7. DLPP assesses bladder; VLPP assesses outlet.
8. BOO: high detrusor pressure with low flow.
9. Incontinence never results from hypermobility alone.
10. Smooth sphincter dyssynergia: autonomic dysreflexia.
11. Dysfunctional voiding is non-neurogenic.
12. Polyuria: >40 mL/kg/24 h.
13. Nocturnal polyuria index >33% in elderly.
14. Terminal dribble: voiding symptom; post-micturition dribble: post-micturition.
15. Suprasacral lesion: DO + DSD.
16. Sacral lesion: acontractile detrusor.
17. Suprapontine lesion: synergic voiding.
18. Maximum cystometric capacity: can no longer delay voiding.
19. Normally silent bladder afferents: C fibres.

10. Bladder underactivity causes overflow incontinence too.

# 106 Evaluation and Management of Males With Urinary Incontinence

*Reading time ≈ 21 min (slow pace)*

## 1 · DEFINITION AND DETERMINANTS

- **Urinary incontinence (UI)** — International Continence Society (ICS): complaint of any involuntary loss of urine; a storage lower urinary tract symptom (LUTS) → *Ch 104*.
- Male continence = external urethral sphincter (EUS, **most important**), internal urethral sphincter, supports and innervation.
- Risk factors: **age (major, independent)**, LUTS, infection, functional/cognitive impairment, neurological disease, obesity, diabetes, prostatectomy, drugs, smoking, alcohol.
- Reversible causes — **DIAPPERS**: Delirium, Infection, Atrophic urethritis, Pharmacological, Psychiatric, Endocrine, Reduced mobility, Stool impaction.
- Male:female ≈ 1:2. **urgency urinary incontinence (UUI) predominates** > stress (SUI) > mixed (MUI). Male UI is more often multifactorial → establish aetiology before treating.

## 2 · TYPES

| Type (ICS) | Mechanism | Male context |
|---|---|---|
| SUI — on effort, cough, sneeze | Abdominal pressure exceeds urethral pressure (sphincter deficiency) | Almost only **after prostatectomy** (EUS damage) |
| UUI — with urgency | Detrusor overactivity (DO) overcomes a normal outlet | ~**90%** have DO on urodynamics; = wet overactive bladder (OAB) |
| MUI | DO + sphincter deficiency | Uncommon; may follow prostatectomy |
| Nocturnal enuresis | Wetting during sleep | New in adult man → suspect **high-pressure chronic retention**, upper-tract dilatation, renal failure |
| Continuous | Fistula, exstrophy/epispadias, ectopic ureter | Rare |
| With chronic retention | Failure to empty; residual ≥**300 mL** | "Overflow" no longer ICS term; spinal cord injury (SCI) ≥T6 → autonomic dysreflexia risk |

- Others: postural, insensible, **postmicturition dribble** (after dressing), functional (normal tract, cannot reach toilet), situational (coital/giggle — rare in men).
- Enuresis = wetting while asleep; nocturia = waking to void. Terminal dribbling is a voiding symptom; postmicturition dribble a postmicturition symptom.

## 3 · EVALUATION

### History and examination

- When (cough, urgency, asleep), how often, how much, precipitants (cold, latch-key), containment (pads, condom catheter), previous prostate/pelvic surgery, neurological and bowel/sexual function, drugs, smoking, alcohol, caffeine, **most bothersome symptom**.
- Examine: abdomen (palpable bladder, scars) → genitalia (meatal stenosis or phimosis → retention) → digital rectal examination (DRE), pelvic squeeze, anal tone and sensation → lower-limb neurology, mobility, cognition → **cough test supine then standing**.

### First-line tests

- **Body mass index (BMI)**: weight loss may help SUI (evidence extrapolated from women). With an artificial urinary sphincter (AUS), abdominal pressure above its balloon (61–70 cm $H_2O$) causes leak through the cuff.

- **Dipstick** in all; abnormal → culture; haematuria → cytology, cystoscopy, upper-tract imaging.
- **Bladder diary**: only fully validated = International Consultation on Incontinence Questionnaire Bladder Diary (ICIQ-BD). **3 days = 7 days**; include a weekend. Gives frequency, volumes, nocturnal polyuria, leaks.
- Questionnaire: International Prostate Symptom Score (IPSS) does **not measure incontinence** → use **ICIQ-UI Short Form** (score 0–21; unscored item 6 localises type). Repeat both after treatment.
- **Pad test**: not routinely recommended; quantifies leakage; cannot separate UUI from SUI.

| Severity | Pads/day | Pad weight/day |
|---|---|---|
| Mild | 1 | <200 g |
| Moderate | 2–4 | 200–400 g |
| Severe | >4 | >400 g |

- Pad weight beats pad count (men change pads for hygiene); neither validated.
- **Flow + ultrasound post-void residual (PVR)**: fast-rise curve → OAB; slow rise, long tail → bladder outflow obstruction (BOO). Flow alone cannot separate BOO from detrusor underactivity. One representative flow acceptable (ICS 2017) — ask him.
- Prostate-specific antigen (PSA) only if life expectancy >10 years and diagnosis would change management. Renal function/glucose if high PVR or diabetes.

### Advanced

- Imaging: high PVR, suspected upper-tract disease, stones. Cystourethroscopy: stricture, BOO, haematuria, high PVR.
- **Urodynamic studies (UDS)** only if the result alters management — conservative and medical therapy first, UDS before invasive treatment (International Consultation on Incontinence [ICI], European Association of Urology [EAU], American Urological Association [AUA]) → *Ch 107*.
- Filling cystometry: DO with leak = DO incontinence (DOI). Cough/Valsalva upright at 200 mL, repeat at capacity → abdominal leak-point pressure.
- SUI = symptom; **urodynamic stress incontinence (USI)** = leak on effort without detrusor contraction; **cough-associated DOI** mimics SUI. Resolve by filling supine (DO less likely) and coughing: leak without DO = USI.

> **KEY POINT** Every man: history, examination, BMI, dipstick, diary, questionnaire, flow, PVR. Optional: PSA, bloods, imaging, endoscopy, invasive UDS — usually just before surgery.

## 4 · URGENCY URINARY INCONTINENCE

**ALGORITHM**
Lifestyle + bladder training + pelvic floor muscle training (PFMT) ≥6 weeks (ideally 3 months) → antimuscarinic or β3-agonist (± combination) → UDS optional → onabotulinumtoxinA / sacral neuromodulation (SNM) / posterior tibial nerve stimulation (PTNS) → augmentation or ileal conduit

- Conservative: cut fluid by **25%** (if intake >1 L/day); avoid caffeine and fizzy drinks; stop smoking; lose weight. Start with drugs for quicker relief.
- **Antimuscarinics**: class-wide similar efficacy; extended-release preferred; trial one agent then move on. **Anticholinergic burden** ≥3 months raises dementia risk ~50%, plus falls. Trospium (quaternary) — little brain penetration. Detail → *Ch 113*.
- **β3-agonists** now co-first-line: similar efficacy, less dry mouth/constipation. Mirabegron — monitor blood pressure. Vibegron — no cytochrome P450 interaction, does not cross blood–brain barrier. **Cognitive impairment → β3-agonist or early surgery**, not anticholinergic.
- Mirabegron + solifenacin beats escalating solifenacin. α-blocker + antimuscarinic or β3-agonist for mixed storage–voiding LUTS. Tadalafil: only approved phosphodiesterase-5 inhibitor for LUTS with erectile dysfunction.

| | OnabotulinumtoxinA | SNM | PTNS |
|---|---|---|---|
| Delivery | Cystoscopic, **100 U** (200 U neurogenic DO) | S3 lead, implanted generator | Needle behind medial malleolus, 30 min weekly × 12 |
| Strength | Repeat 6–9 months | Most effective, durable >10 years | Least invasive; better for dry OAB |
| Drawback | Retention ~**10%** → must accept self-catheterisation | Most invasive, costliest | Frequent visits; no maintenance protocol |

- SNM test: ≥**50%** improvement → permanent generator; technique → *Ch 115*. No head-to-head trials of the three.
- Refractory: containment (pads, catheter) or augmentation enterocystoplasty/ileal conduit — protects upper tracts, high morbidity. Autoaugmentation abandoned.

## 5 · STRESS URINARY INCONTINENCE

**ALGORITHM**
Supervised PFMT ≥3 months (± biofeedback, electrical stimulation; better started pre-op) → ± duloxetine → exclude anastomotic/urethral stricture and bladder dysfunction (UDS) → surgery by severity

- After radical prostatectomy wait **6–12 months** — continence keeps improving. Meanwhile: penile clamp, condom catheter, pads; avoid long-term urethral catheter (traumatic hypospadias).
- **Duloxetine** (serotonin–noradrenaline reuptake inhibitor): off-label in men; short-term benefit, frequent discontinuation.
- **AUS = gold standard** for moderate–severe SUI (~80% continence; Scott 1973 MCQ).
  †Components: urethral cuff, pressure-regulating balloon, scrotal pump; activated about 6 weeks after implantation; deactivate before urethral catheterisation or endoscopy to avoid erosion.
- **Bulking agents**: no durable benefit — do not offer for post-prostatectomy incontinence (PPI).
- **Male slings** for mild–moderate SUI: fixed transobturator (AdVance, under membranous urethra) or adjustable (Remeex, Argus-T, adjustable transobturator male system [ATOMS] cushion). Adjustable: perhaps higher cure, more complications; neither superior.
- **ProACT**: two bladder-neck balloons adjusted via scrotal ports; explant up to half; **contraindicated after radiotherapy**.
- Radiotherapy, severe leakage and prior transurethral surgery worsen fixed-sling results → AUS.
- Neurogenic SUI often needs several procedures (coexistent low compliance, DO) → *Ch 109*.

| EAU 2023 recommendation (PPI) | Grade |
|---|---|
| Offer AUS for moderate–severe PPI | Strong |
| Warn of AUS/ProACT complications, mechanical failure, explantation | Strong |
| Inform of duloxetine side effects, off-label use | Strong |
| Fixed transobturator sling for mild–moderate PPI | Weak |
| Do not offer bulking agents; no ProACT after radiotherapy | Weak |

## 6 · MIXED AND OTHER TYPES

- **MUI**: treat most bothersome component first. SUI may be masked by DO → treat DO, repeat UDS.
- High-pressure chronic retention with enuresis: **catheterise to decompress first**, then relieve BOO (resection, enucleation, prostatectomy).
- Enuresis with low residual → usually OAB → antimuscarinic/β3-agonist ± desmopressin. Neobladder night leakage → conservative.
- Postmicturition dribble: PFMT squeeze after voiding + **urethral milking** (fingers behind scrotum, press forward to penile base, then milk penis, twice).

- ICI and EAU algorithms share one spine: simple assessment → empirical therapy by type → UDS and specialist treatment on failure; **red flags** (haematuria, pain, recurrent infection, pelvic radiotherapy, abnormal DRE, high PVR) go straight to specialist. AUS grade: ICI A.

## 7 · LANDMARK TRIALS

| Trial | Design | Result | Take-home |
|---|---|---|---|
| MASTER | Non-inferiority randomised controlled trial, male sling vs AUS for urodynamic SUI after prostatectomy | Equivalent at 12 months; secondary outcomes favoured AUS; few completely dry | Sling acceptable alternative; counsel realistically |

## 8 · MCQ ONE-LINERS

1. Commonest male UI type: UUI.
2. Most important male continence determinant: EUS.
3. Male SUI cause: prostatectomy.
4. New adult enuresis: exclude high-pressure chronic retention.
5. Significant residual in chronic retention: ≥300 mL.
6. Retention with SCI ≥T6 risks autonomic dysreflexia.
7. ~90% of men with UUI show DO.
8. Only fully validated diary: ICIQ-BD.
9. 3-day diary equals 7-day.
0. IPSS does not measure incontinence.
1. ICIQ-UI-SF score range: 0–21.
12. Severe leak: >4 pads or >400 g/day.
13. Flow cannot distinguish BOO from underactivity.
14. Fluid reduction: 25% if intake >1 L/day.
15. Cognitive impairment: prefer β3-agonist.
16. Mirabegron: monitor blood pressure.
17. OnabotulinumtoxinA OAB dose 100 U; neurogenic DO 200 U.
18. Retention after toxin in men: ~10%.
19. Wait 6–12 months after prostatectomy before SUI surgery.
20. AUS: gold standard, ~80% continence.
21. Bulking agents not for PPI.
22. ProACT contraindicated after radiotherapy.

# 107 Urodynamic and Videourodynamic Evaluation of the Lower Urinary Tract

*Reading time ≈ 41 min (slow pace)*

## 1 · DEFINITION AND ROLE

- **Urodynamics (UDS)**: dynamic study of transport, storage and evacuation of urine — tests used singly or together to diagnose suspected lower urinary tract (LUT) dysfunction and assess the effect of known dysfunction. International Continence Society (ICS) separates UDS *studies* (tests) from UDS *observations* (what is seen).
- LUT has two jobs: **low-pressure storage** (protects kidneys, continence) and **voluntary evacuation**. Failure gives bothersome lower urinary tract symptoms (LUTS) or harm (urinary tract infection (UTI), renal failure) — sometimes silently.
- UDS is useful when history, examination and simple tests are insufficient: (1) to **diagnose the cause of symptoms**; (2) to determine the **impact of a disease that can silently damage the upper tracts** — spinal cord injury (SCI), multiple sclerosis (MS), radiation cystitis.
- Roles: show pathophysiology, predict upper-tract risk and treatment outcome, explain treatment failure.

### Wein functional classification — the frame for every study

| Site | Overactive | Underactive |
|---|---|---|
| Bladder | Failure to store | Failure to empty |
| Outlet | Failure to empty | Failure to store |

Each study is designed to place the patient in this grid: bladder, outlet or both — storage, emptying or both.

### Principles of interpretation

- Decide the questions first → design the study to answer them → customise. Symptoms alone are not an indication.
- Aim to **reproduce the symptom** and identify its cause. An abnormality without symptom reproduction is not necessarily diagnostic; failure to record an abnormality does not exclude it.
- Interpret with history, postvoid residual (PVR) and frequency–volume chart.

> KEY POINT **Box 107.1 — Urodynamic risk factors** (treat even if asymptomatic): impaired compliance; detrusor–external sphincter dyssynergia (DESD); detrusor–internal sphincter dyssynergia (DISD); high-pressure detrusor overactivity throughout filling; detrusor leak point pressure **> 40 cm$H_2O$**; poor emptying with high storage pressure. Each is a route to sustained high intravesical pressure.

## 2 · PREPARATION

### Clinician and patient

- **Medication status** (antimuscarinics, $\beta_3$-agonists) decided and documented in advance: to assess drug effect → test on regular dosing; to uncover aetiology of empirically treated symptoms → stop the drug first.
- **ICS pre-invasive work-up**: validated symptom/bother score, **3-day frequency–volume chart**, free uroflowmetry and PVR. A normal free flow and low PVR beforehand let an abnormal invasive flow be recognised as artefact.
- Explain purpose; well tolerated; commonest complaint afterwards is dysuria.
- **Urinalysis before every invasive study**; symptomatic UTI → postpone until treated.

### Antibiotic prophylaxis (AUA / SUFU Best Practice Policy)

"Index" patient (normal anatomy, no risk factors): **no routine prophylaxis** (level 1). Non-index: **single dose within 1 h** — first choice oral trimethoprim–sulfamethoxazole; alternatives cephalosporin, amoxicillin–clavulanate, aminoglycoside ± ampicillin, fluoroquinolone.

| Prophylaxis YES | Prophylaxis NO |
|---|---|
| Neurogenic LUT dysfunction (NLUTD); bladder outlet obstruction (BOO) or raised PVR; age > 70; **asymptomatic bacteriuria**; immunosuppression (esp. renal transplant); chronic catheters incl. clean intermittent catheterisation (CIC); high-risk orthopaedic implant | Diabetes; **recurrent UTI history**; recent hospitalisation; obesity; menopause; gender; prior urological surgery; cardiac valve disease (not routinely) |

Pattern: "yes" = stasis, colonisation, foreign material, immune compromise, implants. Trap: asymptomatic bacteriuria is treated here, unlike most urological settings.

### Facility

- Quiet private room with space for wheelchairs/hoists; videourodynamics (VUDS) needs fluoroscopy space. Have pessary/Scopette for prolapse reduction.
- Unanswered question → repeat fill/void in the same session.

## 3 · NON-INVASIVE COMPONENTS

### Bladder diary (frequency–volume chart)

- 3 days recommended; gives functional capacity, voiding frequency, nocturnal output. Used to set the cystometric target volume and to judge whether a flow's voided volume is representative.

### Uroflowmetry

- Parameters: voided volume (VV), maximum and average flow (Qmax, Qave), voiding time vs flow time, time to Qmax; **voiding efficiency = VV / (VV + PVR) × 100**.
- **Qmax depends on VV**: linear above 150 mL, hyperbolic below → minimum **VV 150 mL**. Alternatives: corrected Qmax (Qmax/√VV), Siroky nomogram (men), Liverpool nomogram (men and women).
- † Usual norms: Qmax > 15 mL/s in men (< 10 suggests obstruction), > 20 mL/s in women.
- Curve: **bell-shaped** normal; **flattened/plateau** → fixed obstruction (e.g. stricture); **interrupted/straining** → underactivity, obstruction or abdominal voiding.
- Limitation: flow cannot say *why* — low flow is obstruction or weak detrusor; only detrusor pressure separates them.

### Postvoid residual

- By bladder scan or catheter (state method). Shows emptying is impaired, not why. Confirm a raised PVR with a repeat, especially if there was no true desire to void. No agreed "elevated" threshold.

## 4 · EQUIPMENT, TRANSDUCERS AND QUALITY CONTROL

- Minimum: multichannel — vesical pressure (Pves), abdominal pressure (Pabd), subtracted detrusor pressure (Pdet) and flow; optional electromyography (EMG), urethral pressure. Infusate: saline or contrast; **not $CO_2$**.

| Feature | External water-filled | Air-charged | Microtip |
|---|---|---|---|
| Reference level | **Upper border of symphysis pubis**; re-level on position change | None (no hydrostatic effect) | — |
| Problems | Air bubbles; motion and hydrostatic artefact; underdamped | Overdamped (> 3 Hz attenuated) — blunts coughs | Expensive, reusable |
| Standards | Basis of most nomograms (reference standard) | No reference values | — |

- Systems are **not interchangeable** — Pdet may differ by up to 10 $cmH_2O$; patterns transfer, absolute values (pressure–flow, urethral profile) do not.

> **KEY POINT** **Quality control** † (ICS Good Urodynamic Practice): zero to atmospheric pressure; flush lines free of air; check initial resting Pves and Pabd are physiological and similar (Pdet near 0); **cough test** before filling, at intervals and before voiding — equal Pves and Pabd spikes confirm transmission (a biphasic Pdet spike means poor transmission → re-flush/reposition). Common artefacts: rectal contractions (fall in Pdet without cause), catheter displacement or loss of signal, kinked tubing, flow artefact from stream hitting the funnel edge or tapping.

## 5 · FILLING CYSTOMETRY (CMG)

### Technique

- Filling runs from start of infusion to "permission to void" = **maximum cystometric capacity (MCC)** — a strong but not uncomfortable desire; set target from diary and prior PVR.
- **Pdet = Pves − Pabd**. Single-channel Pves cannot tell detrusor from abdominal rises; Pabd by rectal catheter preferred (vaginal/stomal less validated).
- Filling rates: standard adult **30–100 mL/min** (medium); † ICS physiological rate < body weight (kg)/4 mL/min; slow in neurogenic bladders. Neurogenic: physiological rate, body-warm saline (EAU).
- Position that provokes the symptom — DO is commoner upright or on moving from supine.

### What is measured

- **Normal (formerly "stable")**: Pdet near zero throughout, no involuntary contractions.
- **Sensation** — the only subjective parameter: † first sensation, first desire, strong desire, urgency.
- **Capacity**, **compliance**, **involuntary detrusor contractions (IDC)**.

### Detrusor overactivity (DO)

- Urodynamic observation of IDCs during filling, spontaneous or provoked. **Idiopathic DO (IDO)**; **neurogenic DO (NDO)** — relevant neurological disorder; non-neurogenic secondary DO. Terms "hyperreflexia"/"instability" obsolete. Label comes from history — tracings look identical.
- Absence does not exclude it: **up to 50%** of women with urgency urinary incontinence (UUI) show no DO. DO occurs in a minority of asymptomatic volunteers, more on ambulatory UDS.
- Confirm it is involuntary (confused patient may simply void); record volume, pressure, leakage (DO incontinence); repeat at slower fill if uncharacteristic. Characterise: single/multiple, phasic/terminal, suppressible or not.
- Prognosis: in BOO with overactive bladder (OAB), a **single terminal IDC** predicts storage-symptom resolution after transurethral resection of prostate (TURP). In NDO, higher Pdetmax and longer IDCs → upper-tract deterioration.
- **Stress-induced DO** (ICS: cough-associated DO): cough triggers an IDC — mimics stress urinary incontinence (SUI) but is detrusor in origin.

### Compliance

- **Compliance = ΔV / ΔPdet** (mL/$cmH_2O$), between start of filling and MCC (or before a leak-causing contraction), excluding contractions.
- No agreed normal; < 20 mL/$cmH_2O$ proposed as poor. But a small-capacity bladder gives a low number without danger → **absolute storage pressure matters more**: **> 40 $cmH_2O$** associated with upper-tract damage.
- Causes of impaired compliance: neurological (SCI, spina bifida — outlet resistance from DESD, or lower motor neuron (LMN) decentralisation); long-term BOO; radiation; tuberculosis.

| Compliance pitfall | Resolution |
|---|---|
| Rapid filling → accommodation artefact | Stop/slow filling; pressure falls |

| Compliance pitfall | Resolution |
|---|---|
| Sustained low-amplitude IDC mimics low compliance | Stop filling — returns to baseline |
| Pop-off: vesicoureteric reflux (VUR), diverticulum → compliance looks falsely good | VUDS |
| Pop-off: incompetent outlet vents pressure | Occlude outlet; may appear only after sling/artificial urinary sphincter |

## 6 · LEAK POINT PRESSURES

Two independent measurements; both seen only in incontinent patients. Record provocation and site.

| | Abdominal LPP (ALPP) | Detrusor LPP (DLPP) |
|---|---|---|
| Definition | Total intravesical pressure at which leakage occurs from raised abdominal pressure, no detrusor contraction | Lowest Pdet at which leakage occurs without detrusor contraction or abdominal strain |
| Measures | Sphincter strength | Bladder's response to outlet resistance |
| Bad value | **Low** = weak sphincter | **High** = danger to upper tracts |
| Setting | SUI | Low compliance / neurogenic bladder |

### ALPP technique

- Standing; total pressure including baseline; start at **150 mL**, repeat every 50 mL; no leak at capacity → remove urethral catheter and read from rectal line. Valsalva LPP (VLPP) is lower than cough LPP — state method. Larger catheter → lower ALPP; some women and more post-prostatectomy men leak only with catheter out.

| ALPP ($cmH_2O$) | Inference (McGuire) |
|---|---|
| **< 60** | Intrinsic sphincter deficiency (ISD) |
| 60–90 | Equivocal |
| ≥ 90 | Little/no ISD (hypermobility) |

Caution: ISD and hypermobility (Q-tip > 30°) coexist; no hypermobility → SUI must be ISD; ALPP alone poorly predicts surgical success. ICS umbrella term: **urodynamic stress test**.

### DLPP

- McGuire: myelodysplastic children with **DLPP > 40 $cmH_2O$** → hydronephrosis or VUR in 85%. Not a validated safe cut-off (20 $cmH_2O$ more sensitive in children) — target **as low as achievable**.
- Why it matters: bladder pressure is already dangerous before urethral "pop-off". High outlet resistance → less leakage but transmission to kidneys. Treat by lowering bladder pressure (or outlet resistance, e.g. sphincterotomy for DESD) → repeat UDS.

**Occult SUI**: SUI in a continent woman with pelvic organ prolapse (POP) revealed only on prolapse reduction.

## 7 · URETHRAL PRESSURE PROFILOMETRY (UPP)

- Brown–Wickham: perfused side-hole catheter withdrawn slowly; records pressure needed to open the urethra.
- Parameters: maximum urethral pressure; **maximum urethral closure pressure (MUCP)** = max urethral − intravesical; functional length (women ~3 cm; MUCP 40–60 $cmH_2O$ normal).
- **MUCP ≤ 20 $cmH_2O$** → ISD ("type III" SUI), predicted sling/suspension failure.
- Unlike ALPP, UPP does not need or diagnose SUI. ICS: unclear utility — does not discriminate incompetence, grade severity or predict surgical success; EAU: do not use UPP or LPP to grade severity.

## 8 · PRESSURE–FLOW STUDIES (PFS)

- Voiding from permission to void until the patient considers it finished; simultaneous flow; note inhibition from the setting.

| Pdet | Flow | Interpretation |
|---|---|---|
| Low/normal | High/normal | Unobstructed |
| High | Low | **BOO** |
| Low | Low | Detrusor underactivity (obstruction may be hidden) |

- ICS terms: normal detrusor function; **detrusor underactivity** (reduced strength and/or duration → prolonged or incomplete emptying); **acontractile detrusor** ("neurogenic acontractile" replaces "areflexia"); situational inability to void.

### Men — nomograms and indices

Abrams–Griffiths, Schäfer linear passive urethral resistance relation (LinPURR, grades obstruction and contractility), ICS provisional nomogram — all derived from TURP patients and classify similarly.

| Index | Formula | Cut-offs |
|---|---|---|
| **Bladder outlet obstruction index (BOOI)** | PdetQmax − 2Qmax | **> 40 obstructed · 20–40 equivocal · < 20 unobstructed** |
| **Bladder contractility index (BCI)** | PdetQmax + 5Qmax | **> 150 strong · 100–150 normal · < 100 weak** |

- Limitation: BOOI ≥ 40 needs Pdet ≥ 40 even at zero flow → a decompensated detrusor masks obstruction.
- **Stop tests** estimate isovolumetric pressure (Piso): voluntary (lowest value), mechanical, continuous occlusion. In men Piso < 50 $cmH_2O$ suggests underactivity.
- BOO also affects storage: ~2/3 of men with benign prostatic obstruction have DO, resolving in half to two-thirds after relief.
- **Micturitional UPP** (Yalla): localises obstruction — drop > 5 $cmH_2O$ within prostatic urethra.
- MCQ Bethanechol supersensitivity test (Lapides; Cannon's law of denervation): 2.5 mg SC; rise **≥ 15 $cmH_2O$** at 100 mL = positive (neurogenic). Unreliable, rarely changes management (CIC regardless).

### Women

- Women void at low pressure (pelvic floor relaxation, some by straining); no benign prostatic hyperplasia (BPH)-like model → **BOOI/ICS nomogram cannot be used** (grossly underestimate female BOO).
- Suspect with LUTS, incomplete emptying, recurrent UTI, prior anti-incontinence surgery, prolapse.

| Method | Criterion | Comment |
|---|---|---|
| Cut-points | e.g. Qmax ≤ 12 mL/s + PdetQmax ≥ 25 $cmH_2O$ (Defreitas); Chassagne ≤ 15 / ≥ 20 | Anatomic obstruction only; overlap |
| **VUDS (Nitti)** | Radiographic obstruction between bladder neck and meatus with sustained contraction of any magnitude | Localises; works despite weak detrusor |
| **Blaivas–Groutz** | Free Qmax vs Pdetmax; none/mild/moderate/severe | Two separate voids; **overdiagnoses** |
| Solomon–Greenwell | PdetQmax − 2.2Qmax: < 0 unlikely; > 18 almost certain | Includes functional obstruction; newer |

Female underactivity: no universal criteria; associated with reduced sensation.

## 9 · EMG AND SPHINCTER COORDINATION

- ICS standard: two perineal skin-surface electrodes. **Concentric needle**: better signal, uncomfortable, dislodges.
- Normal: EMG quietens and urethral pressure falls **before** Pdet rises; returns after voiding.
- **Guarding reflex**: EMG rises during an IDC to inhibit it — normal, not dyssynergia.

| | **DESD** | **Dysfunctional voiding** |
|---|---|---|
| Cause | **Suprasacral cord lesion** (higher → likelier) | Learned; no neurological lesion |
| Behaviour | Involuntary sphincter contraction during DO/voiding | Intermittent sphincter contraction during voiding |
| Significance | High pressure → poor compliance → upper-tract risk | LUTS; confirm intermittent free flow; fluoroscopy improves accuracy |

- **DISD**: suprasacral lesion above sympathetic outflow (T10–L1); usually with DESD; **VUDS only**.
- **Primary bladder neck obstruction (PBNO)**: in non-neurogenic men, women, children; bladder neck invisible to EMG → needs fluoroscopy.

## 10 · VIDEOURODYNAMICS

- Simultaneous UDS + fluoroscopy — most precise evaluation where anatomy and function interact; **gold standard in NLUTD** (EAU: optimum test; if unavailable, CMG + PFS).
- Adds "where": localises obstruction; only way to assess **bladder neck** (PBNO, DISD); confirms equivocal EMG; shows VUR (volume, pressure, grade) and diverticula; shows incontinence not seen on examination; DLPP in patients who cannot be positioned (tetraplegics).
- **Reflux pop-off**: without fluoroscopy compliance is overestimated — a dangerous bladder looks safe.
- Technique: C-arm or tilting table; SUI views standing; void in usual position; screen only at provocation, pressure rises and voiding (lowest dose).
- PBNO in young men: type I high-pressure/low-flow; type II normal pressure, low flow, narrow neck; type III delayed bladder-neck opening — fluoroscopy critical for II and III.
- Refer to VUDS centre: NLUTD, unexplained retention in women, prior radical pelvic surgery, diversion, renal transplant, pelvic radiation; young men/women without anatomical cause (AUA/SUFU).

## 11 · AMBULATORY URODYNAMICS AND NEWER TESTS

- **Natural filling** with portable recording. Some neuropathic bladders with poor compliance on conventional CMG are normal on ambulatory monitoring — rapid filling overstates pressure. DO is detected more often (including in asymptomatic people).
- † Indication: symptoms (e.g. urgency/UUI) unexplained by repeated conventional UDS.
- Penile cuff test, detrusor wall thickness, near-infrared spectroscopy — none yet replaces subtracted cystometry.

## 12 · INDICATIONS BY CONDITION (AUA/SUFU, EAU)

### Female SUI

- Pure, clinically demonstrated, uncomplicated SUI ("index" patient): UDS may be omitted (AUA/SUFU 2023; EAU: do not routinely perform).
- Perform UDS in non-index: prior anti-incontinence or POP surgery, symptom–examination mismatch, voiding dysfunction, significant urgency/UUI, raised PVR, unconfirmed SUI, NLUTD.
- When SUI not shown with catheter in place → repeat with catheter removed. High-grade POP → stress test with reduction.

### Male LUTS

- PVR and uroflow optional/recommended in initial evaluation; no UDS needed to start an α-blocker.
- AUA/SUFU: **PFS when it matters whether obstruction is present, before invasive, morbid or irreversible treatment**. No obstruction → worse surgical results.
- EAU PFS triggers before invasive treatment: prior failed treatment; cannot void > 150 mL; **Qmax > 10 mL/s**; PVR > 300 mL; age > 80 or < 50. Each marks a man in whom obstruction cannot be assumed.
- **Impaired compliance is the only absolute urodynamic indication to treat BOO.**
- OAB: no UDS in uncomplicated initial work-up; consider in refractory/complicated cases before invasive therapy.

### Women with voiding symptoms

- PFS separates obstruction from impaired contractility; functional obstruction (PBNO, dysfunctional voiding) is invisible on examination/endoscopy → VUDS.

### NLUTD

- Goal: **low-pressure filling** even at the cost of incomplete emptying and catheterisation; only UDS distinguishes DO from poor compliance. RCTs unethical.
- UDS essential before treatment and in follow-up in SCI, myelomeningocele (potentially harmful storage); helpful in BPO with Parkinson disease/MS; usually not needed first in post-stroke UUI.
- Non-invasive testing first (PVR, diary); CMG ± PFS, EMG; VUDS preferred.

## 13 · LANDMARK TRIALS

| Trial | Design | Result | Take-home |
|---|---|---|---|
| ValUE | RCT, 630 women, uncomplicated SUI: office evaluation ± UDS | Success ~77% both arms | Office evaluation alone non-inferior before SUI surgery |
| Dutch embedded RCT | Discordant UDS randomised: immediate mid-urethral sling vs UDS-tailored | Equal cure | UDS discordance need not change surgery |
| SISTEr | Burch vs pubovaginal sling | ALPP/DO did not predict outcome | UDS adds little in stress-predominant SUI |

## 14 · MCQ ONE-LINERS

1. Pdet = Pves − Pabd.
2. Water-filled transducer reference level: upper border of symphysis pubis.
3. Minimum voided volume for reliable Qmax: 150 mL.
4. Plateau flow curve: fixed obstruction such as stricture.
5. Standard adult filling rate: 30–100 mL/min.
6. Compliance = ΔV/ΔPdet; < 20 mL/cm$H_2O$ proposed as poor.
7. Storage pressure > 40 cm$H_2O$ associated with upper-tract damage.
8. DLPP > 40 cm$H_2O$ in myelodysplasia: hydronephrosis/VUR in 85% (McGuire).
9. ALPP < 60 cm$H_2O$ suggests ISD; ≥ 90 suggests hypermobility.
10. VLPP is lower than cough LPP in the same patient.
11. MUCP ≤ 20 cm$H_2O$ defines ISD (type III SUI).
12. Up to 50% of women with UUI show no DO on CMG
13. Single terminal IDC predicts storage-symptom resolution after TURP.
14. BOOI = PdetQmax − 2Qmax; > 40 obstructed.
15. BCI = PdetQmax + 5Qmax; < 100 weak.
16. BOOI cannot be used in women.
17. Blaivas–Groutz nomogram overdiagnoses female obstruction.
18. VUDS (Nitti) female BOO: radiographic narrowing with sustained contraction of any magnitude.
19. Guarding reflex is normal EMG rise during an IDC.
20. DESD requires a suprasacral lesion.
21. DISD and PBNO are diagnosed only by VUDS.
22. Asymptomatic bacteriuria: give prophylaxis before UDS.
23. Prophylaxis of choice: single oral trimethoprim–sulfamethoxazole within 1 h.
24. Impaired compliance: only absolute urodynamic indication to treat BOO.
25. EAU PFS trigger: Qmax > 10 mL/s (not < 10).
26. Bethanechol test positive: rise ≥ 15 cm$H_2O$ (Cannon's law).
27. VUDS is gold standard invasive UDS in NLUTD.

# 109 Neuromuscular Dysfunction of the Lower Urinary Tract

*Reading time ≈ 42 min (slow pace)*

## 1 · PRINCIPLES AND CLASSIFICATION

- Neuropathic lower urinary tract (LUT) dysfunction is largely **predictable from lesion site and nature** (destructive, inflammatory, irritative) — usually, not always. Acute dysfunction may differ from chronic.
- Describe every disorder by **five International Continence Society (ICS) parameters**: sensation (normal/absent/impaired); detrusor (normal, overactive, areflexic, impaired contractility); compliance (normal/↓/↑); smooth sphincter (synergic/dyssynergic); striated sphincter (synergic, dyssynergic, bradykinetic, impaired voluntary control, fixed tone).
- **Neuroplasticity**: structural/neurochemical change reversible up to a point, then fixed. In bladder outlet obstruction (BOO): hypertrophy, ↑ type III:I collagen (↓ compliance), partial denervation, and nerve growth factor (NGF)–driven afferent sensitisation → detrusor overactivity (DO). This is why symptoms persist after urodynamically successful de-obstruction — the lesion starts dysfunction, plasticity maintains it.

| Level | Typical disorders | Detrusor | Smooth sphincter | Striated sphincter |
|---|---|---|---|---|
| Suprapontine | Stroke, tumour, Parkinson disease (PD), dementia, cerebral palsy | Overactive | Synergic | Synergic (± impaired voluntary control; PD bradykinesia) |
| Multiple system atrophy (MSA) | — | Overactive → impaired | **Open** | Denervated |
| Suprasacral cord | Spinal cord injury (SCI), multiple sclerosis (MS), transverse myelitis | Overactive | Synergic (dyssynergic if above T7) | **Dyssynergic** |
| Sacral cord / cauda equina | Sacral SCI, disc, myelodysplasia | Areflexic | Competent non-relaxing; may open | Fixed tone |
| Peripheral | Radical pelvic surgery, diabetes, tabes | Impaired / areflexic | Open (pelvic surgery) | Fixed tone / synergic |

**KEY POINT** Dyssynergia appears only when the lesion lies **between the pons and the sacral cord**; an open bladder neck marks autonomic (MSA, radical pelvic surgery, myelodysplasia) or sacral damage. PD and MSA share the detrusor pattern but **not the outlet**.

## 2 · LESIONS AT OR ABOVE THE BRAINSTEM

### Cerebrovascular accident (stroke)

- Urinary incontinence (UI) persisting 7 days predicts poor survival; commonest type = impaired-awareness incontinence > urgency UI (UUI).
- **Acute**: retention from areflexia ("cerebral shock") → **chronic**: DO with intact sensation and **synergic sphincters**. If the patient can voluntarily contract the striated sphincter → urgency/frequency only; if not (cortex/internal capsule lesion) → UUI. Basal ganglia/thalamic lesions keep voluntary control.
- **True detrusor–sphincter dyssynergia (DSD) does not occur**: apparent "dyssynergia" is **pseudodyssynergia** — electromyographic (EMG) flare from voluntary attempts to inhibit an involuntary contraction.

- Men with pre-stroke LUT symptoms (LUTS): poor flow/high post-void residual (PVR) usually prostatic, but do full urodynamic study (UDS) before outlet surgery to exclude DO with impaired contractility.

### Dementia, brain injury, tumour, others

- **Dementia**: UI from DO or a normal reflex at the wrong time/place (no reason to inhibit). Anticholinergics worsen cognition, delirium, falls and raise new-onset dementia risk → **β3 agonist strongly preferred** (Society of Urodynamics, Female Pelvic Medicine and Urogenital Reconstruction [SUFU]).
- **Traumatic brain injury**: above the pontine micturition centre (PMC) → DO, synergic; isolated brainstem injury below PMC → DSD possible.
- **Brain tumour**: site, not histology (superior frontal lobe) — DO, patient unaware of LUT events; frontal lesions can cause isolated retention.
- **Normal-pressure hydrocephalus**: dementia + gait ataxia + UI (DO); ventriculoperitoneal shunt improves UUI.
- **Cerebral palsy**: most have control; LUTS ~half; DO with synergic sphincters; adults with LUTS — poor compliance/detrusor leak point pressure (DLPP) ≥40 cm $H_2O$ in about half → full evaluation. Conservative care succeeds in ~¾.

### Parkinson disease vs multiple system atrophy

- **PD**: dopaminergic loss in substantia nigra removes basal-ganglia inhibition of the micturition reflex (loss of inhibitory D1, unopposed facilitatory D2). LUTS 35–70%, storage symptoms dominate. UDS: DO; sphincter **bradykinesia** and sporadic activity mimic DSD but are not obstructive.
- Treatment: overactive bladder (OAB) drugs with low central nervous system (CNS) effect (mirabegron, trospium, darifenacin, extended-release fesoterodine); onabotulinumtoxinA; posterior tibial nerve stimulation (PTNS) and sacral neuromodulation (SNM) effective; subthalamic deep brain stimulation improves capacity.
- **transurethral resection of the prostate (TURP) in PD** (transurethral resection of prostate): not contraindicated — old "post-TURP incontinence in PD" cases were MSA. Confirm PD, exclude MSA, do videourodynamics first.
- **MSA**: glial α-synucleinopathy, cell loss includes intermediolateral columns and **Onuf nucleus** → early severe LUTS + erectile dysfunction (ED); DO early, poor emptying late. Treating DO or sphincteric UI worsens emptying → storage + clean intermittent catheterisation (CIC); desmopressin for nocturia. **Prostatectomy hazardous.**

| Feature | MSA | PD |
|---|---|---|
| Urinary symptoms / ED | Precede or accompany parkinsonism; ED near universal | Years after diagnosis; ED ~⅓ |
| Dominant problem; PVR | UI; high PVR | Frequency/urgency; PVR normal |
| Bladder neck | **Open** | Synergic |
| Striated sphincter EMG | **Denervation** (motor-unit) | Normal (bradykinesia) |
| Detrusor | DO → areflexia | DO; areflexia uncommon |
| Outlet surgery | Worsens | Acceptable after work-up |

## 3 · SPINAL CORD DISEASE

### Multiple sclerosis

- Demyelination with axonal sparing, almost always cervical cord. LUTS 50–90%; presenting complaint in up to 15%.

- UDS: DO commonest (~⅔); DSD in 30–65% of those with DO; areflexia/impaired contractility in a quarter; sensation usually intact (helps separate pseudodyssynergia).
- Complication risk factors: **DSD in men, high filling pressure, indwelling catheter**. Upper-tract damage less common than SCI; symptoms do not track disease → **baseline UDS essential**.
- Management: anticipatory, **reversible** (pattern changes over time) — antimuscarinic + CIC cornerstone; onabotulinumtoxinA durable; SNM in selected (magnetic resonance imaging [MRI]-compatible devices). Credé/Valsalva contraindicated; indwelling catheter last resort, suprapubic if used.

## Spinal cord injury

- State column vs cord level: sacral cord begins at column T12–L1; conus → cauda equina ~L2. Stones: risk highest in first 3 months (immobilisation hypercalciuria).
- **Spinal shock**: depressed cord excitability at/below lesion → areflexic, acontractile smooth bladder; bladder neck closed, some striated activity, no voluntary control → retention, overflow only. **Catheterise** (CIC preferred) keeping pressure low. Lasts usually **6–12 weeks** (up to 1–2 years); reflex contractions return with lower-limb deep tendon reflexes.
- **Suprasacral SCI**: DO (C-fibre–mediated reflex emerges through afferent sprouting, NGF) + smooth synergia + striated DSD → storage *and* emptying failure at high pressure. Lesions at or above cord **T7–T8** (column T6) may add smooth sphincter dyssynergia.
- **Sacral SCI**: areflexia, compliance normal then may fall (decentralisation plasticity); competent non-relaxing bladder neck (may later open); fixed striated tone. Silent upper-tract damage can occur **without vesicoureteral reflux (VUR)**. Straining/Credé meets a fixed outlet.

| | **Suprasacral (complete)** | **Sacral (complete)** |
|---|---|---|
| Detrusor | Overactive | Areflexic |
| Compliance | Normal initially | Normal/high → may fall |
| Smooth sphincter | Synergic (dyssynergic above T7–T8) | Competent non-relaxing → may open |
| Striated sphincter | Dyssynergic | Fixed tone, no voluntary control |
| Somatic signs | Spasticity, hyperreflexia | Flaccidity, ↓ reflexes |
| Management | Lower pressure (drugs/toxin) + CIC | CIC + keep compliance safe |

- **Neurology ≠ urodynamics**: somatic completeness ≠ autonomic completeness; occult second levels. DSD occurs only with suprasacral cord lesions, never suprapontine → coordination is supraspinal (pontine). Thoracolumbar (T10–L2 column) lesions least predictable. **Manage by UDS, not by neurological level**; one study may not suffice.
- Upper-tract risk: detrusor fibrosis, leak point pressure ≥75 cm $H_2O$, DO for >⅓ of filling → treat neurogenic DO (NDO) aggressively. †DLPP >40 cm $H_2O$ is the classic threshold for upper-tract deterioration.
- Management (suprasacral): if pressures low or can be made low → treat as emptying failure with CIC; antimuscarinic and/or β3 agonist, intradetrusor onabotulinumtoxinA if refractory. Sphincterotomy or intrasphincteric toxin (men) → convert to storage failure with condom collector, only if unable/unwilling to do CIC.

## Autonomic dysreflexia (AD)

- Potentially fatal emergency in SCI **above T6** (above splanchnic sympathetic outflow); after spinal shock, may appear years later. Noxious stimulus below lesion → unopposed spinal sympathetic reflex (no descending medullary inhibition).
- Features: pounding headache, hypertension, flushing and sweating above lesion, **bradycardia**; may cause cerebral haemorrhage, seizure. Triggers: bladder/rectal distension, blocked catheter, instrumentation, stones, sores, fracture, sex.
- UDS risk: complete, cervical, DSD, poor compliance; defined during UDS as systolic rise ≥20 mm Hg → monitor blood pressure (BP) in all SCI studies.

**ALGORITHM**
†Sit upright, loosen clothing → **remove stimulus** (drain bladder, unblock catheter, rectum) → if BP persists: nitroglycerin 2% paste above lesion (first line) → labetalol / intravenous nitroprusside → prophylaxis: terazosin or prazosin (α1) → intractable: sacral rhizotomy, sympathectomy, cordectomy

- Nifedipine (10–20 mg sublingual during cystoscopy) now banned in many centres — uncontrolled hypotension.

### VUR and women with SCI

- VUR in 17–25% of SCI, driven by high pressure and infection; **chronic urethral catheter worst** for upper tracts. Treat the bladder first (lower storage pressure/outlet resistance); anti-reflux surgery in a thick bladder is hard. VUR artefactually enlarges capacity and lowers pressures on UDS.
- Destroyed urethra: bladder neck closure + suprapubic catheter, or diversion.
- Women: no external collecting device — paraplegics master CIC, tetraplegics often need indwelling catheters.

### Other cord disorders

- **Acute transverse myelitis**: retention almost universal → DO with DSD; manage as SCI; UDS before irreversible therapy.
- **Cervical myelopathy/stenosis**: DO ± DSD, or underactivity (posterior funiculus) → UDS guides therapy.
- **Neurospinal dysraphism**: myelomeningocele ~90% of spina bifida cystica; level correlates poorly with UDS. Typical adult: **areflexic bladder with open bladder neck**, fixed sphincter → leaks at fixed pressure; DSD in 10–15%. Poor compliance, high pressures and DO predict renal decline. Stress UI: bulking safer than sling/artificial urinary sphincter (does not raise closure pressure enough to reduce compliance). Medical + onabotulinumtoxinA → augmentation/diversion.
- **Tethered cord**: presents after growth spurts; no typical pattern (DO commonest); urinary recovery after release lags pain and motor.
- **Sensory neurogenic bladder** (tabes, B12 deficiency/subacute combined degeneration): lost sensation → large capacity, decompensated detrusor; B12 replacement may reverse. **Motor neurogenic bladder** (poliomyelitis): retention with intact sensation.

## 4 · LESIONS DISTAL TO THE CORD

- **Disc disease**: central prolapse (L4–L5, L5–S1) compresses cauda equina. Commonest finding: **normally compliant areflexic bladder**; patients strain or present in retention. Compliance preserved (unlike myelomeningocele) because disc injury is incomplete for preganglionic parasympathetic motor fibres. **Pre-laminectomy UDS** (clinical and medicolegal).
- **Cauda equina syndrome**: saddle anaesthesia, loss of voluntary anal/urethral sphincter control and sexual response; acontractile detrusor. **Decompression within 48 h** gives better urinary and rectal recovery. Sphincter recovers before detrusor.
- **Radical pelvic surgery** (abdominoperineal resection [APR], radical hysterectomy): pelvic plexus injury. Permanent pattern: impaired contractility + fixed striated tone + **open non-functional bladder neck** + ↓ compliance → leak with abdominal pressure or retention; positive bethanechol supersensitivity. Early: **CIC and wait 6–12 months**. **Avoid prostatectomy unless BOO proven**. Nerve-sparing radical hysterectomy reduces long-term urinary symptoms.
- **Herpes zoster** (sacral dorsal root ganglia): retention days–weeks after rash; resolves in 1–2 months. **Herpes simplex**: retention + cerebrospinal fluid (CSF) pleocytosis + sacral radiculitis = **Elsberg syndrome**, transient.
- **Diabetic cystopathy**: sensory then motor neuropathy → ↓ sensation → ↑ capacity → ↓ contractility → ↑ PVR. In men, pressure–flow studies separate it from BOO; "dyssynergia" is straining artefact.

DO also common (early phase/coexisting cerebrovascular disease) → UDS before treatment. Timed voiding prevents distension decompensation.

- **Guillain–Barré syndrome**: ↑ PVR, retention; manage reversibly (CIC) pending recovery.

> **KEY POINT** Distal lesions hit the peripheral reflex arc → areflexia with a non-relaxing/fixed outlet; compliance falls only when preganglionic parasympathetic motor fibres are damaged. Hence urgent decompression, waiting and CIC outrank outlet surgery.

## 5 · SPECIFIC FUNCTIONAL AND MISCELLANEOUS CONDITIONS

### Detrusor–sphincter dyssynergia

- True DSD requires a lesion **between sacral cord and PMC** (SCI, MS, transverse myelitis); without one, doubt the diagnosis (exception: Hinman syndrome in children).
- Pseudodyssynergia: abdominal straining or attempted inhibition of a contraction — misdiagnosis leads to harmful treatment.
- Untreated, ~half of men develop VUR, upper-tract damage, stones, urosepsis; far less in women. DLPP ≥40 cm $H_2O$ after sphincterotomy = failure.
- Treatment: oral sphincter drugs disappoint. **CIC + DO control preferred**; sphincterotomy or sphincteric toxin (poor durability); indwelling catheter; diversion. Urethral stents abandoned.

| Blaivas type | EMG pattern | Lesion | Treatment |
|---|---|---|---|
| 1 | Activity rises with detrusor pressure, relaxes suddenly at peak | Incomplete | Observe unless VUR, hydronephrosis, AD |
| 2 | Sporadic contractions throughout | Complete | Treat |
| 3 | Crescendo–decrescendo, obstruction throughout | Complete | Treat |

### Functional outlet disorders

- **Dysfunctional voiding** (Hinman, "non-neurogenic neurogenic bladder"): involuntary striated sphincter contraction during voiding without neurological disease, proven by pressure–flow–EMG without straining.
- **Non-neurogenic chronic retention**: PVR ≥**300 mL** persisting ≥6 months on ≥2 occasions. High risk (hydronephrosis, chronic kidney disease, recurrent urinary tract infection [UTI], UI with skin sequelae) → drainage; suprapubic preferred to urethral long term.
- **Primary bladder neck obstruction**: young/middle-aged men, long-standing symptoms, normal prostate and cystoscopy. Diagnose by videourodynamics (BOO localised to bladder neck). α-blockers help some; **bladder neck incision definitive**. "Trapped prostate" (Turner-Warwick) → small TURP including bladder neck.
- **Female BOO**: rare; Groutz criteria — free maximum flow (Qmax) ≤12 mL/s + detrusor pressure at Qmax ≥20 cm $H_2O$. Bladder neck surgery risks sphincteric UI.
- **Fowler syndrome**: retention in young women (capacity >1 L without urge), normal MRI, often polycystic ovaries; concentric needle EMG — **complex repetitive discharges and decelerating bursts**; urethral closure pressure >100 cm $H_2O$. Hormones, drugs and toxin fail; **SNM is the treatment** (~70%), and Fowler syndrome predicts SNM response.

### Other conditions — key points

- **Myasthenia gravis**: high incontinence after prostatectomy. **Isaacs syndrome**: sphincter spasm (catheter will not pass) → suprapubic drainage, plasmapheresis. **Wernicke**: DO or poor contractility, resolve with thiamine.
- Myelopathies (tropical spastic paraparesis, schistosomal, syringomyelia) → DO + DSD; root disease (cytomegalovirus, Lyme) → areflexia. **Lupus cystitis**: thick wall, hydronephrosis → steroids.
- **Radiation**: early storage symptoms recover by 6 months; late effects progressive.
- **Defunctionalised bladder**: rehabilitate by cycling; transplant feasible without augmentation.

## 6 · EVALUATION AND RISK STRATIFICATION

- Guidelines: American Urological Association (AUA)/SUFU neurogenic LUT dysfunction (NLUTD) 2021; European Association of Urology (EAU) Neuro-urology 2022. **Primary goal: prevent renal damage.**
- All: history (neurological condition, bladder management, goals), examination (cognition, mobility, **hand dexterity**, spasticity, sensation, skin), urinalysis; PVR if voiding; diaries.
- **Defer full assessment during spinal shock or acute brain injury**; no irreversible therapy in this phase.

| Risk (AUA/SUFU) | Typical | Defining findings |
|---|---|---|
| Low (all) | Suprapontine or uncomplicated peripheral | Spontaneous voiding, low PVR, stable; normal imaging/renal function |
| Unknown | Infrapontine–suprasacral; sacral/infrasacral | Storage + voiding symptoms, ↑ PVR, recurrent UTI; not yet assessed → UDS, upper-tract imaging, renal function |
| Moderate | — | Neurogenic retention; DO with incomplete emptying; normal tracts |
| High (any one) | — | Poor compliance, ↑ storage pressure, DO with DSD, VUR; hydroureteronephrosis, scarring, large stones; abnormal renal function |

**KEY POINT** Low risk needs no routine imaging, renal function or UDS. DO alone fits low risk (suprapontine, synergic); DO in the high-risk row means DO with infrapontine pressure consequences. Risk is judged by **pressure, compliance and upper tracts**, not by diagnosis.

## 7 · MANAGEMENT

- **Goals**: protect upper tracts; control infection; low-pressure storage and emptying; continence and control; avoid catheter/stoma if possible; social acceptability.
- **Principles**: start with simplest, most reversible therapy and climb; combine mechanisms; shared decisions (dexterity, cognition, carers, prognosis).

**ALGORITHM**
Low-pressure storage + regular emptying: CIC + antimuscarinic / β3 agonist → intradetrusor onabotulinumtoxinA → neuromodulation (selected) → augmentation cystoplasty / continent diversion → bladder neck closure + suprapubic catheter / ileovesicostomy / incontinent diversion (last resort)

### Emptying

- **CIC is the gold standard**; keep volumes <**500 mL**. Credé/Valsalva not recommended (outlet resistance → high pressure).
- CIC vs indwelling urethral catheter: less pyelonephritis, epididymo-orchitis, urosepsis. If chronic catheter needed: **suprapubic > urethral** (avoids urethral erosion).

### Storage

- Antimuscarinics first-line for NDO (mirabegron effective); refractory → **intradetrusor onabotulinumtoxinA** (~60% continue long term); warn about CIC.
- DO + DSD: detrusor relaxation + CIC preferred; sphincterotomy only for selected men (high failure, repeats); intrasphincteric toxin not routine.
- Neuromodulation: **sacral deafferentation + anterior root stimulator (Brindley)** in complete SCI — continence, fewer UTIs, stable kidneys, AD resolves; rhizotomy benefits even if stimulator off → *Ch 115*. PTNS for voiding patients with OAB (MS, PD, stroke); SNM for NLUTD without SCI or spina bifida.

- Reconstruction (failed less invasive options, e.g. cannot catheterise): bladder neck closure (retropubic better than perineal) + suprapubic catheter; augmentation + fascial sling; ileovesicostomy.

### Urinary tract infection

- **Treat bacteriuria only if symptomatic.** Correct risk factors (stones, residuals). **No prophylaxis** for CIC or indwelling catheter users. Refractory → upper-tract imaging, cystoscopy, UDS.

### Bladder cancer

- Historically squamous carcinoma with long-term catheters; contemporary data: incidence comparable to general population but often muscle-invasive at diagnosis; catheters and UTI, not SCI, are the risk. **AUA/SUFU 2021: no routine surveillance cystoscopy**; scope for haematuria.

## 8 · FOLLOW-UP

| Body | Schedule |
|---|---|
| AUA/SUFU 2021 (risk-based) | Low risk: none routine. Moderate/high: yearly history, examination, renal function; upper-tract imaging every 1–2 years; repeat UDS for change; bowel reconstruction: yearly electrolytes/acid–base, imaging |
| EAU | Specialist review every 1–2 years; ultrasound + residual 6-monthly; dipstick 2-monthly; yearly examination and bloods |
| American Paraplegia Society (SCI) | Annual for 5–10 years then alternate years; renogram — >20% fall in renal plasma flow → investigate; annual cystoscopy with indwelling catheter (**not endorsed** by AUA/SUFU) |

## 9 · MCQ ONE-LINERS

1. Suprapontine lesion: DO with synergic sphincters; true DSD never occurs.
2. DSD requires a lesion between the PMC and the sacral cord.
3. Smooth sphincter dyssynergia added in SCI at or above cord T7–T8 (column T6).
4. Open bladder neck + denervated striated sphincter = MSA, not PD.
5. Post-TURP incontinence in "PD" historically reflected undiagnosed MSA.
6. Preferred OAB drug in dementia: β3 agonist.
7. Spinal shock usually lasts 6–12 weeks; bladder areflexic, bladder neck closed.
8. Autonomic dysreflexia: SCI above T6; hypertension with bradycardia.
9. First-line drug for acute AD: nitroglycerin 2% paste after removing stimulus.
0. Blaivas type 1 DSD: incomplete lesion, sphincter relaxes at peak detrusor pressure.
1. Untreated DSD: about half of men develop upper-tract complications.
2. Commonest UDS finding in lumbar disc disease: normally compliant areflexic bladder.
13. Cauda equina: decompression within 48 h improves urinary recovery.
14. Radical pelvic surgery: open bladder neck, fixed striated tone, ↓ compliance.
15. Elsberg syndrome: herpes simplex sacral radiculitis with retention and CSF pleocytosis.
16. Classic diabetic cystopathy: ↓ sensation, ↑ capacity, ↓ contractility, ↑ PVR.
17. Typical adult myelodysplastic bladder: areflexic with open bladder neck.
18. Fowler syndrome EMG: complex repetitive discharges and decelerating bursts; treat with SNM.
19. Non-neurogenic chronic retention: PVR ≥300 mL for ≥6 months on ≥2 occasions.
20. CIC volumes should stay below 500 mL.
21. No antibiotic prophylaxis for CIC or indwelling catheter users.
22. AUA/SUFU 2021: no surveillance cystoscopy in NLUTD.
23. Sacral deafferentation with Brindley stimulator abolishes AD and NDO.
24. Primary bladder neck obstruction: definitive treatment is bladder neck incision.

# 110 Overactive Bladder

*Reading time ≈ 24 min (slow pace)*

## 1 · DEFINITIONS AND TERMINOLOGY

- **Overactive bladder (OAB)** — International Continence Society (ICS): "urinary urgency, usually accompanied by frequency and nocturia, with or without urgency urinary incontinence (UUI), in the absence of urinary tract infection (UTI) or other obvious pathology". **Urgency is the defining symptom**; "urge incontinence" became "urgency incontinence" to tie it to that sensation.
- OAB is a **symptom syndrome**; **detrusor overactivity (DO)** is a **urodynamic observation** — involuntary detrusor contractions during filling. Not interchangeable: OAB may lack DO; DO may be asymptomatic or an artefact.

| Term | ICS definition (all patient-reported) |
|---|---|
| Increased daytime frequency | Patient considers they void too often by day; upper normal **7** voids |
| Nocturia | Waking ≥1 time to void; each void preceded and followed by sleep |
| Urgency | Sudden, compelling desire to void, difficult to defer |
| UUI | Involuntary leakage associated with urgency |

- **Bother**: urgency and UUI impair quality of life (QoL) most; **UUI is the most bothersome lower urinary tract symptom (LUTS) in both sexes**. Frequency bothers at ≥**8** daytime voids; nocturia at ≥**2**.
- **Isolated nocturia** usually means nocturnal polyuria or poor sleep, not bladder dysfunction. †Nocturnal polyuria (ICS): night urine >33% of 24-h output in older adults.
- **Mixed urinary incontinence (MUI)** = stress urinary incontinence (SUI) + UUI ("OAB wet"). SUI + "OAB dry" is **not** MUI — describe it. Urinary + faecal = **double (dual) incontinence**. OAB wet: more psychological burden, more DO (almost always in men).

### OAB versus bladder pain syndrome (BPS)

BPS requires bladder-related pain, pressure or discomfort + ≥1 urinary symptom; pain is excluded from OAB. Overlap is large — probably shared afferent hypersensitivity and central sensitisation.

| Feature | OAB | BPS |
|---|---|---|
| Primary fear | **Leakage** | **Pain** |
| Filling sensation | Phasic urgency waves that rise and recede | Steady, accelerating pain |
| Voided volumes | Variable | More consistent |
| Deferral | Limited — leak risk | Possible, at cost of pain |
| Site | Perineum/penile base, urethra/vagina | Suprapubic ± perineal |
| UUI | Yes | No |

## 2 · EPIDEMIOLOGY AND BURDEN

- Adult OAB ≈ **12–29%**; spread reflects definition — loosest (EpiLUTS) highest, strict ICS (EPIC) lowest.
- Women: OAB 1.5–2×, UUI 2–3× men (NOBLE UUI 7.6% vs 2.6%) — the sex gap is in **wet** OAB. Beyond 75: ~40% of men, ~⅓ of women; men overtake women in the oldest. Highest in Black people.
- **Dynamic**: OAB dry → UUI ~1%/year; remission ~6%/year.
- <Half ever tell a clinician (not asked, "must cope", unaware treatment exists). ~**92%** stop the first antimuscarinic within 2 years; half never refill. Reasons: **unmet expectations** > side effects > cost.

- Third-line: onabotulinumtoxinA 100 U most cost-effective; sacral nerve stimulation (SNS) costliest. Screen for unmet social needs (associated with UUI).

## 3 · PATHOPHYSIOLOGY

- **DO is no longer the sine qua non**: only ~half with DO have urgency; only **44–69%** with urgency show DO. No true animal model (animals cannot report urgency).
- **Neural levels**: forebrain (insula, prefrontal cortex) — sensation, volition, storage inhibition; **periaqueductal grey (PAG) and pontine micturition centre (PMC) — main regulatory level**, storage ↔ voiding switch; sacral cord — immature reflex voiding (infants, neurological disease); bladder — urothelium, interstitial cells, micromotions.

| | **Aδ fibres** | **C fibres** |
|---|---|---|
| Structure | Thinly myelinated | Unmyelinated |
| Normal | Predominant; sense volume and contraction | "Silent" — only extreme distension, cold, heat, irritants |
| Pathology | — | Spontaneously active, hypersensitive |

| | **Urothelium-based hypothesis** | **Myogenic hypothesis** |
|---|---|---|
| Site | Urothelium/suburothelium (densest afferent endings) | Detrusor smooth muscle |
| Mechanism | Local stimuli alter receptors/channels → mediators act on afferents | Patchy denervation → receptor upregulation → spontaneous excitation spreading too widely |
| Result | Excess afferent traffic → urgency | Exaggerated **micromotions** → urgency; coordinated → DO |

- **Micromotions**: normal localised contractions, sensed as filling without pressure rise.
- **Cross-talk**: bowel afferents converge with bladder's; constipation and straining worsen storage and voiding symptoms.
- **Failed central nervous system (CNS) filtering**: normally unconscious "afferent noise" perceived as urgency. Also ischaemia–reperfusion, chronic inflammation, altered microbiome.

> **KEY POINT** Final common pathway: more afferent signal reaches awareness — the bladder or a neighbour turns it up, or the CNS fails to turn it down. Hence DO and obstruction correlate only loosely with OAB.

## 4 · AETIOLOGY AND RISK FACTORS

- **Ageing** (clearest), female sex, metabolic syndrome, bowel and hormonal factors.
- **Bladder outlet obstruction (BOO)**: the standard animal DO model, but in humans **indirect** — OAB prevalence is similar in women (BOO rare) and men, and DO often persists after prostate surgery. BOO and OAB are probably parallel consequences of ageing.
- **Psychological**: depression, anxiety (serotonergic, hypothalamic–pituitary–adrenal pathways; central sensitisation).
- **Behaviour**: "just in case" voiding, straining; coping (toilet mapping, fluid restriction) sustains a negative bladder perception even after objective improvement.

## 5 · CLINICAL ASSESSMENT

- **Minimum** (American Urological Association/Society of Urodynamics, Female Pelvic Medicine & Urogenital Reconstruction [AUA/SUFU]; European Association of Urology [EAU] concurs): **history, examination, urinalysis** → empirical treatment. EAU adds questionnaires and a bladder diary.
- **History**: symptoms and bother; SUI; voiding symptoms (BOO); haematuria; fluid/caffeine; occult neurology (new OAB with erectile dysfunction or tremor, spinal disease); pelvic surgery,

radiotherapy, bowel (faecal incontinence ~**50%** of women with OAB); drugs; polyuria (diabetes, sleep apnoea, lithium).

| Drug class | Relative contraindications |
|---|---|
| Antimuscarinics | Uncontrolled closed-angle glaucoma; cognitive impairment; retention; impaired gastric emptying |
| β3-adrenergic agonists | Uncontrolled hypertension; arrhythmia |

- **Examination**: frailty (Timed Up and Go Test), cognition, abdomen, leg oedema (nocturia). **Pelvic/genital examination is the most important part** — prolapse, vaginal atrophy, stress test; digital rectal examination (DRE) in men; perineal sensation, anal tone.
- **Urinalysis mandatory**; culture if infection suspected (dipstick misses low counts); microhaematuria → full evaluation.
- **Postvoid residual (PVR)**: not needed before conservative or oral therapy if uncomplicated. Measure if retention risk, voiding symptoms (men), prior outlet surgery, neurological disease.
- **Bladder diary**: 3–7 days (minimum 24–48 h). OAB = frequent small voids. **Maximum voided volume = functional capacity**. Sensation grades 0–4: 0 = "just in case" void (behaviour, not OAB); 2–3 urgency; 4 urgency with leak.
- **Patient-reported outcome measures**: International Consultation on Incontinence Questionnaire (ICIQ) modules pair symptoms (ICIQ-OAB) with QoL (ICIQ-OABqol); AUA/SUFU recommend Urogenital Distress Inventory (UDI-6), Incontinence Impact Questionnaire (IIQ) or OAB-q. MCQ OAB-V8 = screening tool, not severity score.

## 6 · URODYNAMICS (UDS)

- **Not** in initial work-up (nor cystoscopy, ultrasound). **Indicated** when the result decides an invasive or irreversible treatment, after multiple failures, or for suspected BOO (OAB after an outlet procedure). In the index woman (low PVR, no prolapse or neurology) UDS improves satisfaction, not outcome.
- OAB findings: **DO** or **increased bladder sensation**. Stop antimuscarinics to show DO; continue them to explain refractory symptoms.
- DO in OAB dry: 60–70% men, 40–50% women; OAB wet: **>90% men**, but absent in 30–40% of women (SUI misread as urgency). DO correlates **best with UUI, worst with nocturia**.
- Technique: provoke (running water, standing), test upright; low-volume DO masking SUI → refill supine.
- **Always assess emptying**: each **10 mL** PVR raises catheterisation risk after onabotulinumtoxinA by **27%**; BOO relief does not guarantee cure — DO persists in up to **60%** after transurethral resection of prostate (TURP).

## 7 · MANAGEMENT

- Chronic, benign → **no treatment is acceptable** after counselling; incontinence-associated dermatitis justifies more aggressive care. Immobility-driven leakage needs mobility, not bladder, treatment.
- Set goals and **realistic expectations** — improvement, not cure; vital before invasive therapy. The large placebo response shows the power of education.
- AUA/SUFU "lines" are **not a compulsory sequence**; often combined. Allow **4–12 weeks** before judging any therapy.

**ALGORITHM**
Confirm OAB, exclude mimics → education → conservative → drugs → tibial nerve stimulation / sacral neuromodulation / onabotulinumtoxinA → augmentation or diversion; reassess at every step

### Conservative

- Fluid reduction ~**25%** helps (avoid under-hydration); caffeine reduction improves urgency and frequency, not consistently UUI.
- **Weight loss** reduces SUI and UUI (†PRIDE (Program to Reduce Incontinence by Diet and Exercise) randomised trial, obese women); bariatric surgery: each 5% lost → ~7–8% lower incontinence risk.
- **Pelvic floor muscle therapy (PFMT) and bladder training**: leaks ↓ **50–80%**, negligible risk.

### Drugs *→ Ch 113*

- Antimuscarinics (extended-release preferred) or β3 agonists (e.g. mirabegron); response is idiosyncratic — switch or combine before invasive work-up.
- **Dementia**: antimuscarinic use >**3 months** → 10–65% higher risk (SUFU) → prefer β3 agonists or trospium, darifenacin, fesoterodine.
- Benign prostatic hyperplasia (BPH) + OAB: α1-blocker + antimuscarinic beats either; retention up to ~3% → caution if PVR raised.

### Third-line *→ Ch 115*

After conservative and drug failure, or earlier if those are unworkable. Frailty predicts functional decline and 1-year morbidity up to 50% even after minor procedures.

| | Tibial nerve stimulation | OnabotulinumtoxinA | SNS |
|---|---|---|---|
| Efficacy | 60–80% respond | UUI ↓59%; ~¼ dry | 5-year success 70–80% |
| Drawback | 12 weekly visits; response by weeks 8–10 | UTI ~20%; catheterisation 0–12% in trials, but real-world retention ~20% women, ≤35% men | Revision >30% at 5 years; battery change |
| Risk | Lowest | Moderate | Highest |

- **Augmentation or diversion**: rarely; weigh against pads or indwelling catheter.
- **International Consultation on Incontinence pathways**: men — treat the outlet alongside the bladder (add α-blocker if BOO); women — treat the most bothersome component of MUI first; frail elderly — comorbidity, function and carer goals first, then a cautious antimuscarinic trial.

> **KEY POINT** Escalate by invasiveness and reversibility, not severity alone: OAB is benign, so patient goals, frailty and PVR decide how far to climb.

## 8 · MCQ ONE-LINERS

1. Defining symptom of OAB: urgency.
2. Upper normal daytime voids (ICS): 7.
3. Most bothersome LUTS in both sexes: UUI.
4. OAB is a symptom diagnosis; DO is urodynamic.
5. SUI + OAB dry is not MUI.
6. Urinary + faecal incontinence: double incontinence.
7. OAB fear: leakage; BPS fear: pain.
8. Highest prevalence study: EpiLUTS; lowest: EPIC.
9. Antimuscarinic discontinuation by 2 years: ~92%.
10. Urgency patients showing DO: 44–69%.
11. Silent afferents awakened in pathology: C fibres.
12. Main regulatory level: PAG and PMC.
13. Bladder-centric theories: urothelium-based and myogenic.
14. Faecal incontinence in women with OAB: ~50%.
15. Most important examination component: pelvic/genital.
16. Diary grade 0: convenience void.
17. DO correlates best with UUI, worst with nocturia.
18. Each 10 mL PVR: 27% higher catheterisation risk after toxin.
19. DO persists after TURP in up to 60%.
20. PFMT cuts incontinence episodes 50–80%.
21. Antimuscarinic dementia risk: beyond 3 months.
22. Minimum therapeutic trial: 4–12 weeks.
23. Nocturia becomes bothersome at ≥2 episodes.
24. Sex gap in OAB lies mainly in UUI.
25. Each 5% weight lost after bariatric surgery: ~7–8% lower incontinence risk.
26. Most cost-effective third-line: onabotulinumtoxinA 100 U; costliest: SNS.

# 111 Underactive Detrusor

*Reading time ≈ 23 min (slow pace)*

## 1 · DEFINITIONS AND SYMPTOMS

- Emptying = contraction against outlet resistance. Drugs and surgery can lower resistance but cannot yet strengthen the detrusor, so **detrusor underactivity (DUA)** management is largely drainage — preferably intermittent self-catheterisation (ISC) when postvoid residual (PVR) is raised.
- **DUA** (International Continence Society [ICS] 2002, urodynamic): "a contraction of **reduced strength and/or duration**, resulting in prolonged bladder emptying and/or failure to achieve complete bladder emptying within a normal time span". **No thresholds** are specified.
- **Acontractile detrusor**: no contraction. Distinguish the **"bashful bladder"** — failure to void during a urodynamic study (UDS) only, often with a history of inability to void in public toilets.
- **Underactive bladder (UAB)** (ICS 2018, symptomatic): "slow urinary stream, hesitancy and straining to void, with or without a feeling of incomplete bladder emptying, sometimes with storage symptoms". Unlike overactive bladder (OAB, anchored on urgency), UAB has **no pathognomonic symptom** and overlaps with OAB and bladder outlet obstruction (BOO).
- Clinically significant PVR is not agreed; one proposal: >**40%** of functional capacity (voided volume + PVR).

| Phase | Symptoms |
|---|---|
| Voiding | Weak stream, intermittency, hesitancy, straining |
| Post-micturition | Incomplete emptying |
| Storage | Frequency and nocturia, **or** reduced desire with infrequent voiding |
| Very large PVR | Incontinence, especially in sleep |

> **KEY POINT** **Sensation decides the picture.** Absent sensation → infrequent voider; if emptying is poor, overflow incontinence worse at night (urethra relaxes in sleep). Preserved sensation → frequent voider with incomplete emptying and nocturia. DUA may also be asymptomatic.

## 2 · EPIDEMIOLOGY

- Population prevalence is **unknown** — no non-invasive marker. In men, voiding LUTS (lower urinary tract symptoms), low flow and high PVR may equally be BOO; in women BOO is rare (2.7–8% of those referred), so the same picture suggests DUA.
- In UDS series: men <50 **9–28%**; men >70 ~**48%**; women 12–45%; highest in the institutionalised elderly, often with **detrusor hyperactivity with impaired contractility (DHIC)** — involuntary contraction emptying <half the bladder. Criteria differ, so figures cannot be pooled.
- DUA often coexists with other dysfunction in the elderly: ~half of men also have detrusor overactivity (DO) or BOO; ~¾ of women DO or stress urinary incontinence (SUI) → **UDS before LUTS surgery in the elderly**.
- Natural history: men with non-neurogenic DUA followed 10 years conservatively showed **minimal progression**.

## 3 · AETIOPATHOGENESIS

Confirmed causes: neurological disease/injury and diabetes. Probable: BOO, ageing, atherosclerosis. Each cause breaks one link of the voiding reflex loop.

| Mechanism | Site | Result |
|---|---|---|
| Myogenic | Myocyte, extracellular matrix (ECM) | Weak contraction despite normal drive |

| Mechanism | Site | Result |
|---|---|---|
| Efferent | Sacral cord, roots, pelvic nerves | Impaired activation |
| Afferent | Peripheral afferents, spinal columns | **Early termination** of the voiding reflex |
| Brain | Pontine micturition centre (PMC), periaqueductal grey (PAG), limbic, prefrontal | Failed integration |

- **Myogenic**: ECM fibrosis impairs force transmission; Elbadawi **degeneration pattern** on electron microscopy (disrupted myocytes, axonal degeneration). Similar fibrosis in asymptomatic elderly women → ageing contributes, not BOO alone.
- **Afferent**: bladder afferents sense volume and contraction, urethral afferents sense flow; both sustain the reflex. Sensation falls with age (reduced insular response on functional magnetic resonance imaging [MRI]).
- **"Stiff balloon" hypothesis (Smith)**: fibrosis → stiff bladder → wall stress rises abruptly → delayed first sensation then urgency → small void → wall stress collapses → afferent drive fades → contraction stops early → raised PVR.

| Category | Causes |
|---|---|
| Neurogenic | Stroke (early), multisystem atrophy, multiple sclerosis, Guillain–Barré, herpes zoster, tabes dorsalis, diabetes, disc prolapse, cauda equina, spinal stenosis, spinal cord injury (SCI), pelvic fracture |
| Myogenic | BOO, diabetes, pelvic arterial ischaemia |
| Iatrogenic | Radical prostatectomy/hysterectomy, rectal resection; radiotherapy |
| Functional | Fowler syndrome; dysfunctional voiding |
| **Drugs (reversible)** | Anticholinergics (antihistamines, antipsychotics, tricyclics, antiparkinsonian), **opioids** |

- †**Fowler syndrome**: young women in retention with a non-relaxing striated sphincter (abnormal sphincter electromyography); responds to sacral neuromodulation.
- **BOO**: animal model — compensation (hypertrophy) → **decompensation** (fibrosis, weak contraction); permanent if not relieved first. Proposed mechanism: high wall tension (Laplace) → ischaemia–reperfusion each cycle → free radicals → denervation. But in men followed >10 years detrusor pressure at maximum flow (Pdet@Qmax) did not fall — prolonged BOO need not decompensate. Most men in acute retention have preserved detrusor function.
- **Diabetes** (diabetic cystopathy): autonomic neuropathy → poor sensation, plus myogenic change. **Temporal theory**: early — osmotic diuresis → compensatory hypertrophy, storage symptoms; late — oxidative damage → poor sensation and emptying.

| Condition | DUA | Key point |
|---|---|---|
| Stroke | ~Half retain acutely | "Cerebral shock"; chronic sequel is DO |
| Parkinson disease | <20% | DO commoner; DUA drug-related |
| **Multisystem atrophy** | **52–95%** | Efferent parasympathetic atrophy; retention; mistaken for Parkinson |
| Multiple sclerosis | ~20% | Lumbosacral plaques |
| Radical pelvic surgery | Hysterectomy: ~72% have lower urinary tract dysfunction | Pelvic plexus injury; nerve-sparing reduces; many recover by 1 year |
| Guillain–Barré, zoster | — | Reversible |

## 4 · DIAGNOSIS

- **Invasive UDS is the only accepted method**; no consensus criteria; almost all measure strength, not speed or duration.
- Usual: low maximum flow (Qmax) + low Pdet@Qmax. Pitfalls: (1) the **bladder outlet relation** (Hill equation) makes pressure and flow inverse, so Pdet@Qmax underestimates strength; (2) outlet resistance is ignored — low flow may be BOO, normal flow may reflect a low-resistance outlet. Hence isovolumetric estimates.

| Measure | Formula | Thresholds / note |
|---|---|---|
| **Bladder contractility index (BCI)** (Abrams) | Pdet@Qmax + 5Qmax | >150 strong; 100–150 normal; **<100 weak**; male-derived |
| Projected isovolumetric pressure (PIP) (Schäfer) | Pdet@Qmax + K·Qmax (K 5 men, 1 older women) | <50 very weak; 50–100 weak |
| Detrusor contraction coefficient (DECO) | BCI/100 | <1 weak |

- †Companion: **BOO index** = Pdet@Qmax − 2Qmax; >40 obstructed, <20 unobstructed.
- **Stop tests** (voluntary or mechanical) and **continuous occlusion** measure isovolumetric pressure directly; voluntary reads ~20 cm $H_2O$ lower (reflex detrusor inhibition); occlusion is most reliable but painful — research only. Criteria diagnose DUA in 5–55% of the same population.
- **Speed**: fall in shortening velocity precedes fall in Watts factor — early marker. **Duration**: in the definition but unmeasured in practice.
- **Failure to void on UDS**: bashful bladder vs true acontractility → history; if doubt, **ambulatory UDS** — over four-fifths of apparently acontractile patients contract.

## 5 · MANAGEMENT

Goals: symptoms, quality of life; prevent infection, stones, reflux with upper-tract damage, overflow skin damage.

**ALGORITHM**
Evaluate (diary, digital rectal examination, urinalysis, flow, PVR; neurological deficit → spinal MRI; stop offending drugs; treat constipation) → timed/double voiding → lower outlet resistance where relevant → ISC for high PVR (suprapubic catheter if ISC impossible) → selected: sacral neuromodulation, outlet surgery, diversion

| Aim | Routine | Experimental | Not recommended |
|---|---|---|---|
| Drainage | Timed/double voiding; catheterisation; diversion | — | Credé, Valsalva, reflex voiding |
| ↓ Outlet resistance | Pelvic floor biofeedback; α-blockers; outlet surgery | Sphincter botulinum toxin | — |
| ↑ Detrusor function | Sacral neuromodulation; anterior root stimulator | Intravesical prostanoids; intravesical electrotherapy; myoplasty | **Muscarinic agonists; anticholinesterases** |

### Conservative and catheterisation

- **Credé/Valsalva** only with an incompetent sphincter (neurogenic); otherwise high pressure → vesicoureteric reflux and reflux into prostate.
- **Biofeedback** helps dysfunctional voiding — a non-relaxing sphincter obstructs flow and reflexly inhibits the detrusor.
- **ISC preferred** (needs cognition, dexterity): fewer infections than indwelling catheters; urethral bleeding in ~⅓. †Frequency set to keep catheterised volumes below ~500 mL. Avoid long-term urethral catheter; suprapubic is the best long-term indwelling option.

### Drugs → *Ch 113*

- **Bethanechol, distigmine**: poor evidence (no overall PVR benefit); side effects include bronchospasm, diarrhoea, salivation and **rare fatal cardiac arrest** → not routine. Response needs intact sensation and muscle — useless with fibrosis or detrusor loss.
- **α-Blockers**: routine; α-blocker + parasympathomimetic improved flow in a single-blind randomised trial.
- Prostanoids ($PGE_2$ contracts detrusor and relaxes urethra; also uterus) — experimental. Novel oral agents (ASP8302, TAC-302) investigational.

### Electrical stimulation → *Ch 115*

- **Sacral neuromodulation**: inhibits urethral afferents that block bladder afferents → restores sensation and contraction. Best for DUA with sphincter/pelvic floor disorder; needs some detrusor function; potential in DHIC (treating DO worsens DUA and vice versa).
- **Sacral anterior root stimulator (Brindley)**: complete SCI; needs intact efferents and muscle (reflex contractions on filling); with sacral rhizotomy.
- **Intravesical electrotherapy (Katona)**: activates Aδ afferents; needs some afferent and muscle function; mainly children; only sham-controlled trial negative.

> **KEY POINT** Every therapy needs a surviving link: Brindley needs efferents and muscle; neuromodulation and electrotherapy need afferents and muscle. **Preserved sensation predicts response** (bethanechol, toxin, electrotherapy).

### Botulinum toxin (sphincter)

Relaxes the sphincter to remove reflex detrusor inhibition or aid straining. 50–100 U cystoscopic (men) or periurethral (women). ~Half of idiopathic cases improve; best with poor sphincter relaxation and good sensation. Short-lived, unlicensed.

### Surgery

| | DUA, low PVR, symptomatic | DUA, catheter-dependent |
|---|---|---|
| Reasoning | Emptying at low Pdet → BOO unlikely significant | Lower resistance enough to void |
| Evidence | Transurethral resection of prostate (TURP) at ~11 years: no benefit over conservative care | Flow rarely normalises, but many void |
| Verdict | **Surgery unlikely to help** | Consider in young, fit patients |

- Poor outcome in retention: voiding pressure **<45** cm $H_2O$, age **>80**, PVR **>1500 mL**.
- Women: bladder neck incision/resection not recommended (incontinence, stenosis).
- **Diversion**: continent catheterisable stoma (young women with idiopathic retention refractory to neuromodulation); ileal conduit for renal deterioration from intramural ureteric obstruction.
- Investigational: **latissimus dorsi detrusor myoplasty** (most acontractile patients voided; complications in ⅓); rectus myoplasty; autologous muscle-derived stem cells. Reduction cystoplasty abandoned.

## 6 · MCQ ONE-LINERS

1. DUA definition (ICS 2002): reduced strength and/or duration.
2. ICS definition of DUA has no numerical thresholds.
3. UAB has no pathognomonic symptom.
4. Failure to void only during UDS: bashful bladder.
5. DUA in men >70 undergoing UDS: ~48%.
6. Elderly with DO plus weak contraction: DHIC.
7. Untreated non-neurogenic male DUA at 10 years: minimal progression.
8. "Stiff balloon" hypothesis: Smith.
9. Degenerative condition with DUA and retention: multisystem atrophy.
10. Stroke: acute retention; chronic DO.
11. BCI = Pdet@Qmax + 5Qmax; weak <100.
12. Apparent acontractility contracting on ambulatory UDS: >80%.
13. Not recommended in DUA: muscarinic agonists, anticholinesterases, Credé.

4. Bethanechol: rare fatal cardiac arrest.
5. Significant PVR proposal: >40% of capacity.
6. Preferred drainage: ISC; long-term indwelling: suprapubic.
7. Strongest predictor of treatment response: preserved sensation.

18. Brindley stimulator needs intact efferents and detrusor.
19. Outlet surgery fails if voiding pressure <45, age >80, PVR >1500 mL.
20. TURP in low-PVR DUA: no benefit at 11 years.

# 112 Nocturia

*Reading time ≈ 47 min (slow pace)*

## 1 · DEFINITION AND TERMINOLOGY

- **Nocturia** — International Continence Society (ICS) 2019: the number of times urine is passed during the **main sleep period**, from the time a person intends to sleep until the time they intend to rise. The period is set by the **individual's own schedule** (shift workers included), not the clock.
- It does **not exclude 1 void per night** and does **not incorporate bother**.
- ICS 2002 said "wake at night **to void**", implying urge-driven waking differs from poor sleep. ICS 2019 dropped the cause: urge and poor sleep are intertwined, and sleep or "non-urological" causes are **leverage for interdisciplinary care, not grounds to exclude** a patient.
- Guidance comes from the European Association of Urology (EAU; standalone 2017 guideline and nocturia sections in Male and Female lower urinary tract symptoms [LUTS] guidelines) and ICS. The American Urological Association (AUA) has none; its overactive bladder (OAB) guideline calls **isolated nocturia a separate entity**.

### Key diary terms

| Term | Definition / formula | Meaning |
|---|---|---|
| Actual number of nightly voids (ANV) | Voids within the main sleep period | Severity count |
| First uninterrupted sleep period (FUSP) | Sleep onset → first night void (h) | Key sleep measure |
| Nocturnal urine volume (NUV) | All night voids **+ first morning void** | Volume made during sleep |
| Nocturnal polyuria index (NPi) | NUV / total 24-h volume (TUV) | Proportion made at night |
| Nocturnal urine production (NUP) | NUV / hours of sleep period | Hourly rate |
| Maximum voided volume (MVV); nocturnal MVV (NMVV) | Largest void in 24 h; in sleep period | Proxies for functional capacity |
| Nocturia index (Ni) | NUV / MVV | >1: night output exceeds capacity |
| Modified Ni (MNi) | NUV / NMVV | Night data only |
| Predicted nightly voids (PNV) | Ni − 1 | — |
| Nocturnal bladder capacity index (NBCi) | ANV − PNV | >0: voids below capacity |

**Trap**: the first morning void is **split** — its volume goes into NUV, but it counts as daytime frequency, not a nocturia episode.

## 2 · WHY NOCTURIA MATTERS

- **Common**: up to **3 in 5** people >70 years void ≥2 times nightly; age is the most robust correlate. Boston Area Community Health survey MCQ: higher in Black and Hispanic people, partly via socio-economic status.
- **Bothersome**: most with ≥**2** voids are bothered, but one void can be very bothersome — **any nocturia that brings a patient to a urologist is clinically significant**. It may dominate the interview and mask other LUTS.
- **Sleep**: urge-driven waking is the **commonest cause of adult sleep disruption**. Slow-wave sleep (N3), the deepest and most restorative stage, clusters in the early night and is not recovered later; most first voids occur within **2–3 h** of sleep onset, so they land in it. Non-rapid eye movement sleep is ≈75% of sleep, in 4–6 cycles nightly, but slow-wave sleep is only 10–25%. Hence daytime fatigue

even when total sleep time is preserved, and a shorter time to first void means worse whole-night sleep.

- **Falls and hip fracture**: nocturia is among the LUTS most strongly linked; older adults are most affected and least steady in the dark.
- **Systemic disease and mortality**: nocturia may herald cardiometabolic (strongest evidence), renal, endocrine or neurological disease. ≥2 voids per night is associated with higher mortality; causality unproven — probably via falls, poor sleep and comorbidity.

> **KEY POINT** Nocturia matters because of when it strikes and what it signals: the first void breaks restorative slow-wave sleep, and the symptom may be the visible edge of systemic disease.

## 3 · PATHOPHYSIOLOGY — PRODUCTION, CAPACITY AND SLEEP

Aetiology is multifactorial — **"Sleep CALM"**: **S**leep disorders, **C**omorbidities, **A**ctions (behaviour, lifestyle), **L**ower urinary tract dysfunction (LUTD), **M**edications. Idiopathic nocturia is small but real.

- **Common mechanism**: every factor upsets the balance of **night urine production**, **functional capacity** and **sleep architecture**.
- Production is continuous; after emptying at bedtime, volume climbs to capacity, and then there are only two endpoints — **wake to void or enuresis**. Sleeping through requires night production ≤ capacity.
- **Sleep, the third arm**: sleep disorders cause "convenience" voids while already awake, may raise production and lower the arousal set-point. The relation is **bidirectional**.

### Nocturnal polyuria (NP) and global polyuria

| Criterion | Threshold | Comment |
|---|---|---|
| NPi (ICS) | >**20%** young to >**33%** older adults | Most used; poor discrimination |
| NUP (ICS alternative) | >**90 mL/h** | More specific for production-driven nocturia |
| Global polyuria | >**40 mL/kg/24 h** (>2.8 L at 70 kg) | Uncommon, but serious differential — **always investigate** |

- **Krimpen study** [MCQ]: NPi >33% in 92% of older men with ≥2 voids but also 70% of men **without** nocturia.
- Excess night production contributes to **most adult nocturia**. NP, global polyuria and small capacity are not mutually exclusive and often coexist.

### Functional capacity and the mismatch

- True capacity = voided volume + post-void residual (PVR); MVV/NMVV are proxies. No consensus: normal usually >300–400 mL; **150–300 mL** suggests diminished capacity — either **24-h** (all voids small) or **night-specific** (small night voids, normal day voids; partly convenience voids).
- Waking depends on the **ratio** of production to capacity: **Ni is the strongest diary determinant of severity and treatment response**. PNV = Ni − 1 because the last night volume leaves as the first morning void.

**ALGORITHM**
NUV 800 mL, MVV 200 mL → Ni = 4 → PNV = 3 → diary shows 6 night voids → NBCi = **3** (voiding well below capacity)

Investigate diminished night capacity when NBCi >**1.3**; NBCi >**2** is strongly associated with severe nocturia.

## 4 · THE BLADDER DIARY

The guideline cornerstone, objectively showing the production–capacity–sleep relationship. EAU: recommended for female LUTS and for male LUTS with prominent storage symptoms or nocturia (strong).

**ALGORITHM**
Measuring container → time and volume of every void → time, volume and type of every drink → mark bedtime (intending sleep) and rising → urgency grade and leakage per void

- **Frequency–volume chart** = void times and volumes only; **bladder diary** adds intake, pads and incontinence.
- International Consultation on Incontinence diary urgency codes: **0** no need (social void), **1** normal desire, **2** urgency that passed, **3** urgency, dry, **4** urgency with leak.
- **Diary ANV is the gold standard for severity**; patients usually overestimate on questionnaires. **Intent governs**: lying awake intending to sleep is still the sleep period.
- **FUSP is the most important sleep measure**: inversely related to night voids, and its prolongation marks successful treatment with better sleep and daytime function — an outcome measure and target.
- Duration: EAU **3 days** (Male: at least 3). Longer diaries are more reliable but adherence falls ("diary fatigue"); completed by fewer than half of urology patients.
- **Nocturnal-only diary** gives ANV, FUSP, NUV, NUP, NMVV and MNi (reasonable surrogate for Ni), but **cannot diagnose global polyuria** — the diagnosis most tied to serious disease. Partial work-around: hourly, the global polyuria threshold is 90 mL/h at 54 kg but 167 mL/h at 100 kg, so very high night volumes in heavy patients may signal 24-h polyuria. Unvalidated; not recommended for women by EAU; better only than no diary.

**KEY POINT** Every cause works through one equation: production against capacity, gated by sleep. The diary measures both sides, and their ratio (Ni) predicts severity better than either alone.

## 5 · EVALUATION — THE SLEEP CALM WORK-UP

History, examination and basic tests in all → tailored ancillary tests and multidisciplinary referral. A validated questionnaire helps at baseline and follow-up; **TANGO** (Targeting the individual's Aetiology of Nocturia to Guide Outcomes) screens all domains, including falls.

| Factor | History / examination | Tests (basic → ancillary) |
|---|---|---|
| Sleep | Sleep habits, apnoea, restless legs, pain, anxiety, depression | STOP-BANG, sleep diary → polysomnography, sleep specialist |
| Comorbidities | Full history; blood pressure, volume overload, neurological signs | Metabolic panel → glycated haemoglobin (HbA1c), electrocardiogram (ECG), echocardiography, natriuretic peptide, albumin:creatinine ratio (ACR), calcium, thyroid, water deprivation test |
| Actions | Fluid, alcohol, caffeine, sodium; body mass index (BMI) | Fluid intake diary |
| LUTD | Concurrent LUTS, prior surgery; abdominal, genital, rectal examination | Urinalysis, PVR → prostate-specific antigen (PSA), uroflowmetry, cystoscopy |
| Medications | Reconcile drugs and **dose timing** | — |

- **Trap**: the urological row is one of five; stopping there misses four-fifths of the differential.
- First-line management = **optimise every Sleep CALM abnormality found**.

## 6 · SLEEP DISORDERS

**Obstructive sleep apnoea (OSA)** is the best-described link: incidence rises with nocturia frequency, and frequent nocturia predicts severe OSA.

| Arm | OSA mechanism | Continuous positive airway pressure (CPAP) |
|---|---|---|
| Production (main) | Hypoxia → pulmonary vasoconstriction → ↑ right atrial transmural pressure → ↑ **atrial natriuretic peptide (ANP)** → natriuresis | ↓ night production |
| Capacity | Intermittent hypoxia → oxidative stress → detrusor instability | Less detrusor overactivity |
| Sleep | More light sleep, less slow-wave sleep | ↑ slow-wave sleep |

- CPAP reduces nocturia by up to ≈**2 voids** in moderate-to-severe OSA — among the **largest effects of any intervention**. EAU Female (strong): suspected OSA → sleep clinic.
- **STOP-BANG**: Snoring, Tired, Observed apnoea, Pressure (hypertension), BMI >**35**, Age >**50**, Neck >**40 cm**, Gender male; ≥**3** = high risk; highly sensitive for moderate-to-severe OSA.
- Establish the patient's **primary motive for getting up** (urge versus already awake). Ask about choking arousals, witnessed apnoeas and morning headache (OSA); an evening urge to move legs relieved by movement (restless legs); and odd night events (parasomnias).
- **Melatonin** may suit older adults, who are most vulnerable to hypnotics, anticholinergics and antidiuretics (limited evidence). Behavioural insomnia treatment improved sleep and reduced night voids. Involve sleep experts when indicated.

> **KEY POINT** OSA is the model for the whole chapter: one disorder raises production, lowers capacity and fragments sleep — which is why treating it gives one of the largest nocturia responses.

## 7 · COMORBIDITIES

- Medical disease acts mainly through **excess night production**. Nocturia may be the **first sign** of hypertension, metabolic syndrome, coronary disease, diabetic end-organ damage or early kidney disease. EAU: consider systemic causes in all; even **non-bothersome nocturia merits a frequency–volume chart**.
- **Volume overload**: fluid third-spaced into the legs by day is mobilised on lying down, adding to NUV — exaggerated in heart failure, where diuresis reduced nocturia with oedema.
- **Hypertension**: blood pressure normally dips **10–20%** in sleep; **non-dipping** probably mediates the link via renin–angiotensin–aldosterone dysregulation → **nocturnal natriuresis**. Hydrochlorothiazide improved nocturia in men with α-blocker-refractory LUTS.

### Disorders of water homeostasis

**Arginine vasopressin (AVP)**: supraoptic and paraventricular nuclei → posterior pituitary → released mainly by rising osmolality → **V2 receptors** on collecting-duct principal cells → water reabsorption. Defects show on the diary as **24-h polyuria**.

| Condition | Defect | Causes |
|---|---|---|
| Central diabetes insipidus (DI) | ↓ AVP secretion | Brain tumour, haemorrhage, trauma, autoimmune |
| Nephrogenic DI | Renal AVP insensitivity | Drugs, electrolyte disturbance, renal failure |
| Primary polydipsia | AVP suppressed by intake | Idiopathic; psychiatric |

Arginine-stimulated **copeptin** (a cleavage product of the vasopressin precursor) appears superior to the water deprivation test. Every global polyuria needs evaluation to avoid missing a neoplastic, traumatic, inflammatory or psychiatric process; cause-specific treatment normalises output.

> **KEY POINT** Comorbid disease reaches the bladder mostly through the kidney — third-spaced fluid, nocturnal natriuresis and failed antidiuresis all raise night production. Global polyuria and non-bothersome nocturia are both reasons to look beyond urology.

## 8 · ACTIONS — LIFESTYLE AND BEHAVIOUR

- Trial evidence is thin and epidemiology inconsistent (patients self-restrict), but physiology is strong — **timing matters more than total amount**: a water bolus causes diuresis within ≈1 h; caffeine and alcohol within 2–3 h (caffeine half-life 2.5–4.5 h) and both disturb sleep; a protein-rich evening meal causes hyperfiltration for 2–3 h.
- **Practice**: restrict fluid, caffeine and alcohol from **3–4 h** before sleep; older adults should drink more by day to avoid dehydration. Patients with high baseline intake gain most.
- Also: **sodium** restriction; weight loss and exercise; **compression stockings** (less daytime leg fluid to mobilise); bladder training, pelvic floor muscle training (PFMT), treating constipation.

## 9 · LOWER URINARY TRACT DYSFUNCTION

- Nocturia accompanies bladder outlet obstruction (BOO), OAB (up to **85%** of trial participants), detrusor underactivity, neurogenic bladder and the genitourinary syndrome of menopause (GSM) → *Ch 110*.
- **Isolated nocturia is not a LUTS** and is not expected to respond to bladder or prostate drugs; use them when LUTD is genuinely suspected.
- Assessment: urinalysis in all (treat infection, repeat); **PVR in both sexes**; PSA only if it changes management. †Urodynamics is not routine, only when LUTD needs defining → *Ch 107*.

### Benign prostatic obstruction

- Relieving BOO should help by lowering PVR and afferent input. Nocturia data are secondary endpoints of benign prostatic hyperplasia (BPH) trials: **α-blockers** carry most of the effect; 5α-reductase inhibitors (5ARIs) alone are weakest. Gains are fractions of a void with a large placebo response — **statistical, not clinical, significance**. Drugs and dosing → *Ch 113*.
- Phosphodiesterase type 5 inhibitors (PDE5Is): EAU — **do not offer** for nocturia.
- **Surgery**: transurethral resection of the prostate (TURP) reduced nocturia more than microwave therapy, α-blocker or watchful waiting, but sleep and quality-of-life gains are inconsistent. Newer techniques (photoselective vaporisation, Aquablation, Rezum, UroLift, prostatic artery embolisation) are not proven better than TURP for nocturia. **Counsel** that nocturia may respond sub-optimally, especially when it is the most bothersome symptom.

### Overactive bladder

- Night symptoms are among the most refractory; daytime response does not predict night benefit. Antimuscarinics may reduce nocturnal urgency and raise night capacity. Tolterodine (night voids as primary endpoint) was negative overall but reduced urge-related night voids; **fesoterodine** (only trial with nocturnal urgency as primary endpoint) was positive. Patients voiding below capacity (**raised NBCi**) respond best to an evening dose.
- β3-agonists (mirabegron, vibegron): marginal reduction (≈0.1–0.2 voids vs placebo); none studied with nocturia as primary outcome.
- **Behavioural therapy** should precede drugs and matches or beats them for nocturia. Adding drugs adds little; group classes or video training cut cost.
- Third-line: onabotulinumtoxinA beats placebo by a fraction of a void; percutaneous tibial nerve stimulation beats sham and ≈ tolterodine; sacral neuromodulation data sparse. Nocturia never a primary outcome.
- Antimuscarinics are on the **Beers Criteria** list for ≥65 years — the core nocturia population.
- GSM (LUTS plus genital and sexual symptoms of oestrogen deficiency): nocturia via LUTD, night production and poor sleep; vaginal oestrogen may help, ospemifene potentially.

| Therapy | EAU Male LUTS | EAU Female LUTS |
|---|---|---|
| α1-blocker | Offer for nocturia with LUTS (weak) | — |

| Therapy | EAU Male LUTS | EAU Female LUTS |
|---|---|---|
| 5ARI | Offer with LUTS and prostate >**40 mL** (weak) | — |
| phosphodiesterase type 5 inhibitor (PDE5I) | **Do not offer** | — |
| Antimuscarinic | Offer with OAB (weak) | Offer after counselling (**strong**) |
| PFMT; vaginal oestrogen | — | Offer (**strong**); offer (weak) |

**KEY POINT** Lower urinary tract drugs act on capacity and outflow, but most nocturia is driven by production — so effects are statistically significant but clinically small. Treat the mechanism, not the organ.

## 10 · MEDICATIONS CAUSING NOCTURIA

| Mechanism | Drugs |
|---|---|
| ↑ Urine output | Diuretics; selective serotonin reuptake inhibitors (SSRIs; block antidiuretic hormone); **calcium channel blockers** (oedema, ↑ ANP); tetracycline; lithium (↓ aquaporin-2); **sodium-glucose cotransporter 2 (SGLT-2) inhibitors** (glycosuria) |
| Insomnia / central nervous system | Stimulants; α-blockers, β-blockers, methyldopa; salbutamol, theophylline, decongestants; corticosteroids, thyroxine; SSRIs, monoamine oxidase inhibitors; carbidopa; phenytoin |
| Bladder toxicity | Ketamine; tiaprofenic acid; cyclophosphamide |

The three groups mirror the production, sleep and bladder arms. Note α-blockers both treat nocturia and may cause insomnia. Reconcile drugs and dosing times; first adjust timing or frequency, or substitute.

## 11 · MANAGEMENT ALGORITHM AND REFRACTORY NOCTURIA

**ALGORITHM**
History + examination + diary + basic tests → identify Sleep CALM factors → optimise them (multidisciplinary) → conservative measures → antidiuretic or timed diuretic in selected refractory patients — diary at every step

- **Multicomponent therapy**: causes interact, so treat them together (EAU **"Shared Care Pathway"**).
- **Target night volume even when capacity is small**: excess production best predicts frequency and response; small-capacity patients improved as NUV fell with capacity unchanged.

### Desmopressin

- AVP normally rises in sleep; a **blunted rise** occurs in some NP patients.
- **Desmopressin** (1-deamino-8-D-arginine vasopressin, DDAVP): selective **V2 agonist** — antidiuretic without pressor effect. The **only US-approved nocturia drug**: **≥2 voids per night due to NP**. Level 1b evidence.

| | Nasal spray | Orally disintegrating tablet (ODT) |
|---|---|---|
| Dose | 1.66 mcg if <65, no risk; ≥65 or at risk → **0.83 mcg** | **Sex-specific**: women **25 mcg**; men **50 mcg** |
| Timing | ≈30 min before bed | Sublingual ≈60 min before bed |

- **Women are more sensitive**: the V2 receptor gene is on the **X chromosome**. In men 75 mcg added hyponatraemia, not efficacy.
- †Advise minimal fluid from 1 h before until 8 h after the dose. †Contraindicated with past or current hyponatraemia, polydipsia, loop diuretics or systemic/inhaled glucocorticoids, estimated

glomerular filtration rate <50 mL/min, syndrome of inappropriate antidiuretic hormone and heart failure.

- **Hyponatraemia** risk factors (sodium <**130 mmol/L**): **age ≥65**, baseline sodium <135, creatinine clearance <60 mL/min, doses above approved. Most occur within **3 weeks**.
- **Sodium monitoring**: EAU — baseline → **day 3** → day 7 → 1 month → periodically; US label — baseline → within 7 days → ≈30 days. EAU Female: avoid if baseline sodium low. A proposed protocol from pooled tablet data — baseline sodium in all, stop if <135 mmol/L, repeat at 7 and 30 days only if ≥65 or at risk — would have prevented every case below 130 at approved doses.
- **Low-dose rationale**: the median effective dose is far below that for maximal antidiuresis, so low doses still work and avoid antidiuresis outlasting sleep.
- EAU Male: desmopressin for NP <65; **low dose** >65 with benefit–risk discussion. **Age 65 is the pivot.**

### Timed diuretics

- Furosemide acts for 6–8 h. Evening dosing **causes** NP; **morning or early-afternoon** dosing treats it (level 1b) by completing natriuresis before bed.
- Population links between diuretics and nocturia are mostly confounding by volume-overload disease. Small trials (bumetanide 4–6 h, furosemide 40 mg 6 h before bed) consistently cut voids. Monitor sodium.
- **Trap**: afternoon furosemide + bedtime desmopressin worked in a trial, but loop diuretics are a **contraindication** to desmopressin in US labelling.
- **Future**: phenotype night urine — predominant water diuresis → desmopressin; predominant natriuresis → afternoon diuretic.

> **KEY POINT** Refractory nocturia is treated by moving urine production out of the night. Either way the price is sodium balance, so dose, age and monitoring define safe use.

## 12 · LANDMARK TRIALS

| Trial | Design | Result | Take-home |
|---|---|---|---|
| MTOPS | Doxazosin, finasteride, combination, placebo | Doxazosin and combination beat placebo; finasteride did not | α-blocker drives benefit |
| VA Cooperative | Terazosin, finasteride, combination, placebo | Only terazosin separated | 5ARI alone does not help |
| CombAT | Tamsulosin, dutasteride, combination | Combination −0.5 vs −0.3/−0.4 | Significant but small |
| Fesoterodine randomised controlled trial (RCT) | Nocturnal urgency primary endpoint | Positive | Helps urgency-driven night voids |
| Nasal spray phase III | 0.83/1.66 mcg vs placebo | Both beat placebo | Basis of approval |
| ODT sex-specific RCTs | Women 25; men 50/75 mcg | Positive; 75 mcg more hyponatraemia | Sex-specific dosing |

## 13 · MCQ ONE-LINERS

1. ICS 2019 nocturia: voids during the main sleep period, any cause.
2. First morning void: volume in NUV, counted as daytime frequency.
3. Mnemonic for causes: Sleep CALM.
4. NP (ICS): NPi >33% in older adults, or NUP >90 mL/h.
5. Global polyuria: >40 mL/kg/24 h.
6. Nocturia index: NUV / MVV.
7. NBCi = ANV − PNV; >2 severe.
8. Most important diary sleep measure: FUSP.
9. CPAP can reduce nocturia by ≈2 voids.
10. Only US drug approved for nocturia: desmopressin.
11. ODT doses: women 25 mcg, men 50 mcg.
12. EAU sodium checks: baseline, day 3, day 7, 1 month.
13. Loop diuretics contraindicate desmopressin.
14. Most first voids occur within 2–3 h of sleep onset.

5. Commonest cause of adult sleep disruption: nocturia.
6. Gold standard severity measure: diary-recorded ANV.
7. Krimpen study showed NPi discriminates poorly.
8. Nocturnal-only diary cannot diagnose global polyuria.
9. OSA raises night output via atrial natriuretic peptide.
10. STOP-BANG cut-offs: BMI 35, age 50, neck 40 cm.
11. Normal nocturnal blood pressure dip: 10–20%.
22. Calcium channel blockers cause nocturia via oedema.
23. PDE5Is are not offered for nocturia.
24. Isolated nocturia is not a LUTS.
25. 5ARI recommendation needs prostate >40 mL.
26. V2 receptor gene lies on the X chromosome.
27. Main hyponatraemia risk factor: age ≥65.
28. Diuretic timing for NP: morning or early afternoon.
29. Investigate small night capacity when NBCi >1.3.
30. Copeptin testing is superseding water deprivation for DI.

# 113 Pharmacologic Management of Lower Urinary Tract Storage and Emptying Failure

*Reading time ≈ 19 min (slow pace)*

## 1 · FRAMEWORK

Drugs are filed by the Wein functional scheme → *Ch 104*: **storage failure** — relax the bladder/↑ capacity or ↑ outlet resistance; **emptying failure** — ↑ detrusor contraction (weak tools) or ↓ outlet resistance. Most lower urinary tract (LUT) drug failures are of **selectivity/therapeutic window** — the same receptor in heart and vessels caps the dose.

## 2 · ANTIMUSCARINICS (OVERACTIVE BLADDER, OAB; DETRUSOR OVERACTIVITY, DO)

† Detrusor contraction is mediated by **M3** receptors although **M2** predominate numerically. Antimuscarinics act mainly in the **storage phase** (↓ urgency, ↑ capacity — partly by damping afferent signalling), not by blocking voiding contraction, so retention is uncommon at clinical doses.

| Agent † | Usual dose | Point to remember |
|---|---|---|
| Oxybutynin | 5 mg 2–3×/day; extended release (ER); transdermal | Highest dry mouth (active metabolite N-desethyloxybutynin); transdermal bypasses first-pass → less dry mouth; most central nervous system (CNS) penetration — avoid in elderly |
| Tolterodine | 2 mg bd; ER 4 mg od | Non-selective; bladder > salivary in vivo |
| Fesoterodine | 4–8 mg od | Prodrug of tolterodine's active metabolite; flexible dose |
| Solifenacin | 5–10 mg od | Moderately M3-selective; long half-life |
| Darifenacin | 7.5–15 mg od | Most M3-selective; constipation |
| Trospium | 20 mg bd | **Quaternary amine** — minimal blood–brain barrier crossing; renally excreted |
| Propiverine, imidafenacin | 20 mg; 0.1–0.2 mg bd | Mixed action / Japan |

- † **Side effects by receptor**: dry mouth (M3 salivary), constipation (M3 gut), blurred vision (M3 ciliary), tachycardia (M2), cognitive impairment (**M1**, brain).
- † **Contraindications**: **uncontrolled narrow-angle glaucoma**, urinary retention, gastric retention, myasthenia gravis, severe ulcerative colitis/toxic megacolon.
- † **Elderly**: cumulative **anticholinergic burden** is associated with cognitive decline and dementia → prefer β3-agonist or non-CNS-penetrant agent; avoid oxybutynin (Beers criteria).
- † Men with bladder outlet obstruction (BOO): safe if post-void residual (PVR) is low; check PVR before and during treatment.

**ALGORITHM**
† OAB: behavioural therapy (bladder training, fluids, pelvic floor) → antimuscarinic *or* β3-agonist (β3 first in elderly/cognitive risk) → switch or combine → onabotA 100 U / sacral neuromodulation / percutaneous tibial nerve stimulation → augmentation cystoplasty

## 3 · B3-ADRENOCEPTOR AGONISTS

† β3 stimulation relaxes the detrusor during storage (↑ cyclic adenosine monophosphate) without the antimuscarinic side-effect profile — no dry mouth, no cognitive burden.

| Agent † | Dose | Cautions |
|---|---|---|
| Mirabegron | 25 → 50 mg od | Avoid in **severe uncontrolled hypertension** (≥**180/110** mm Hg); monitor blood pressure; moderate CYP2D6 inhibitor (metoprolol, desipramine); monitor digoxin |

| Agent † | Dose | Cautions |
|---|---|---|
| Vibegron | 75 mg od | No titration; no clinically relevant CYP2D6 inhibition or blood-pressure effect in trials |

## 4 · COMBINATION THERAPY

Different mechanisms justify an antimuscarinic + β3-agonist, especially when antimuscarinic dose gives intolerable adverse events (AEs) or insufficient response.

| Direction | Evidence | Agents |
|---|---|---|
| Mirabegron added to failing antimuscarinic | SYNERGY, SYNERGY II, BESIDE, MILAI | **Solifenacin only** |
| Antimuscarinic added to failing mirabegron | MILAI II (52 weeks) | Solifenacin 5, propiverine 20, imidafenacin 0.2, tolterodine ER 4 mg |
| Vibegron + antimuscarinic | Phase IIb (vibegron 100 + tolterodine 4 mg) | Benefit extrapolated; no trial after monotherapy failure |

**KEY POINT** Combination beats solifenacin 5 mg and either monotherapy; in non-responders it **equals** solifenacin 10 mg but with less dry mouth — it gains **tolerability, not efficacy**, over dose escalation. No change in heart rate, blood pressure, QT or PVR; constipation roughly doubles. Benefit irrespective of age ≥65 or ≥75.

- **α-blocker + desmopressin**: for nocturia resistant to first-line therapy — ≈30% of men ≥60 with benign prostatic hyperplasia (BPH) have nocturnal polyuria, which α-blockade does not treat. Evidence uncontrolled.
- **Vaginal oestrogen + antimuscarinic**: positive signals weak and sequence-dependent (antimuscarinic after oestrogen); simultaneous-combination randomised controlled trials (RCTs) null; vaginal estradiol ≈ oxybutynin. Additive benefit **not established**.
- † Systemic oestrogen worsens incontinence — do not use for urinary incontinence (UI).

## 5 · INTRADETRUSOR ONABOTULINUMTOXINA (ONABOTA)

† Blocks presynaptic acetylcholine release (cleaves SNAP-25) and reduces afferent signalling; cystoscopic injection into the detrusor.

| | Idiopathic OAB (refractory) | Neurogenic DO (NDO; spinal cord injury, SCI; multiple sclerosis, MS) |
|---|---|---|
| Dose | **100 U** † (≈20 sites) | **200 U** † (≈30 sites) |
| Trial | EMBARK — anticholinergic-refractory only | DIGNITY — equal efficacy in anticholinergic users and non-users |
| Retention | † Raised PVR/need for clean intermittent catheterisation (CIC) — higher in NDO non-catheterisers (MS); patient must accept CIC before treatment | |

- † Commonest AE: urinary tract infection (UTI) — treat active UTI first; check PVR ≈2 weeks after. Effect lasts ≈6–9 months; repeat when it wanes (≥12 weeks apart).
- † Caution: myasthenia gravis, aminoglycosides.
- Adding oral drugs to prolong onabotA effect is common but unevaluated.

## 6 · NOCTURIA — DESMOPRESSIN †

- Vasopressin V2 agonist → ↓ nocturnal urine production; indicated for **nocturnal polyuria**. Low-dose sublingual formulations: women 25 µg, men 50 µg.
- **Hyponatraemia** is the key risk, rising with age **≥65**: check sodium at baseline, within 1 week and at 1 month; stop if low.
- Contraindications: hyponatraemia, polydipsia, heart failure, syndrome of inappropriate antidiuretic hormone, renal impairment, concurrent loop diuretics.

## 7 · INCREASING OUTLET RESISTANCE (STRESS URINARY INCONTINENCE, SUI)

† Pelvic floor muscle training and surgery remain first-line for SUI; no drug is a standard of care. α-agonists fail because systemic agents lack urethral selectivity — vascular α-receptors raise blood pressure before urethral closure improves.

| Drug | Mechanism | Status |
|---|---|---|
| Duloxetine † | Serotonin–noradrenaline reuptake inhibitor at **Onuf's nucleus** → ↑ pudendal tone of striated sphincter | 40 mg bd; nausea → frequent discontinuation; approved in Europe, not USA |
| Phenylpropanolamine, midodrine, norepinephrine | Systemic α-agonists | Weak evidence; hypertension, haemorrhagic stroke → phenylpropanolamine banned by US Food and Drug Administration (FDA) |
| Imipramine † | Tricyclic: anticholinergic + α-agonist-like outlet effect | Mixed UI, enuresis; orthostatic hypotension, cardiac arrhythmia — caution in elderly |
| Clenbuterol | β-agonist | Small positive RCT, unbalanced baseline; tremor; abandoned |
| PSD503 vaginal phenylephrine gel | Local α-agonist | ↓ leakage but poor acceptability; abandoned |

## 8 · FACILITATING EMPTYING

- † **Bethanechol** (muscarinic agonist): **no convincing controlled evidence** of improved emptying in detrusor underactivity; cholinergic AEs (sweating, bradycardia, bronchospasm, diarrhoea). Contraindicated in asthma, peptic ulcer, bradycardia, mechanical obstruction.
- † **α-blockers** (tamsulosin, alfuzosin, silodosin, doxazosin, terazosin): ↓ smooth-sphincter/prostatic tone — BPH, primary bladder neck obstruction, neurogenic LUT dysfunction; aid trial without catheter after acute retention.
- † Striated sphincter: baclofen, benzodiazepines, dantrolene — limited efficacy; sphincteric onabotA.

**Drugs that cause retention †**

Anticholinergics (incl. antihistamines, tricyclics, antipsychotics, antiparkinsonian drugs); α-agonist decongestants (pseudoephedrine); opioids; benzodiazepines; anaesthesia/epidurals. Ask about them in every retention.

## 9 · FAILED DRUGS

| Agent | Indication | Reason for failure |
|---|---|---|
| Vitamin D3, elocalcitol | UUI/OAB | ↓ incontinence only; no urodynamic effect |
| Calcium antagonists (verapamil, nimodipine) | OAB | No effect; cardiovascular users show no bladder impairment |
| $K^+$ channel openers (ZD0947, flupirtine) | OAB | Dose capped by heart; flupirtine hepatotoxic |
| URO-902 (big potassium [BK] channel gene therapy) | OAB | Phase 1 only |
| RimabotulinumtoxinB | Toxin A-refractory DO | Short duration (≤3 months) |
| Intraprostatic onabotA, PRX302, NX-1207 | BPH-related lower urinary tract symptoms (LUTS) | = sham injection — large placebo effect |

## 10 · LANDMARK TRIALS

| Trial | Design | Result | Take-home |
|---|---|---|---|
| SYNERGY / II | Phase III, 12 weeks / 12 months | Solifenacin 5 + mirabegron 50 > monotherapy for UI | Combination effective and safe long-term |
| BESIDE | Solifenacin 5 non-responders | Add-on > solifenacin 5, = solifenacin 10 | Alternative to dose escalation |
| MILAI II | 52 weeks, mirabegron non-responders | Four antimuscarinic add-ons safe, effective | Add antimuscarinic to mirabegron |
| DIGNITY | Pooled RCTs, NDO | OnabotA 200 U equal in anticholinergic users/non-users | Efficacy independent of anticholinergic |
| EMBARK | RCTs, refractory OAB | OnabotA 100 U effective | Standard dose for idiopathic OAB |

## 11 · MCQ ONE-LINERS

1. Receptor mediating detrusor contraction: M3.
2. Most M3-selective antimuscarinic: darifenacin.
3. Quaternary amine, minimal CNS penetration: trospium.
4. Transdermal oxybutynin: less dry mouth — avoids N-desethyloxybutynin.
5. Fesoterodine shares tolterodine's active metabolite.
6. Absolute antimuscarinic contraindication: uncontrolled narrow-angle glaucoma.
7. Cognitive side effects: M1 blockade.
8. Mirabegron caution: severe uncontrolled hypertension.
9. Mirabegron inhibits CYP2D6.
0. Mirabegron add-on evidence exists only with solifenacin.
11. Combination vs solifenacin 10 mg: equal efficacy, less dry mouth.
12. OnabotA dose: 100 U idiopathic OAB, 200 U NDO.
13. Commonest onabotA AE: UTI.
14. Desmopressin key risk: hyponatraemia, especially age ≥65.
15. Duloxetine acts at Onuf's nucleus.
16. Commonest duloxetine AE: nausea.
17. Phenylpropanolamine banned: haemorrhagic stroke.
18. Bethanechol: no convincing evidence for detrusor underactivity.
19. Imipramine: anticholinergic plus outlet α-effect.
20. Intraprostatic toxins failed against sham injection.
21. RimabotulinumtoxinB limitation: short duration.
22. Flupirtine halted for hepatotoxicity.

# 115 Electrical Stimulation and Neuromodulation in Storage and Emptying Failure

*Reading time ≈ 18 min (slow pace)*

## 1 · EVOLUTION AND MECHANISM

- Direct detrusor/sphincter stimulation failed — painful, only locally effective, and bladder and sphincter contracted together. **Indirect stimulation via afferents** needs less current and treats the whole organ → "neuromodulation".
- Milestones [MCQ]: sacral neuromodulation (SNM) ~1985 (Tanagho, Schmidt); first tibial nerve stimulation 1983 (McGuire); percutaneous tibial nerve stimulation (PTNS) derived from acupuncture point **SP6**; first tibial implant 2006.
- **Exclude anatomical causes first** (stones, cancer, fistula, poorly compliant bladder, stricture, prostatic enlargement, prolapse): stimulation acts only on neural control, never on anatomy or muscle bulk.
- Normal voiding: external urethral sphincter (EUS) relaxes, detrusor contracts 1–2 s later; the pontine micturition centre (PMC) acts as an on-switch, gated by EUS relaxation.
- **Current theory (forebrain)**: sacral roots are mixed; stimulation excites afferents → forebrain areas of awareness and alertness → less PMC firing in overactive bladder (OAB). The older efferent theory is less plausible: benefit needs no pelvic floor contraction, and purely sensory nerves work equally.
- **Paradox**: the same therapy treats underactive bladder — raised awareness of filling lets the PMC switch on; in retention SNM works mainly by **striated sphincter relaxation**, not detrusor contraction.

## 2 · SACRAL NEUROMODULATION

### Indications

- **Third-line** for idiopathic OAB after lifestyle/pelvic floor training and drugs (American Urological Association [AUA]/Society of Urodynamics, Female Pelvic Medicine and Urogenital Reconstruction [SUFU] and European Association of Urology [EAU] identical).
- Pre-treatment: frequency–volume diary (OAB definition ignores excess intake); exclude urinary tract infection (UTI), significant post-void residual (PVR), stones, cancer. Urodynamic detrusor overactivity (DO) **does not predict** success.
- **Non-obstructive urinary retention (NOUR)**: chronically high PVR needing drainage; PVR is the only objective outcome. Fowler syndrome predicts response; increasing age lowers odds.
- †Faecal incontinence is an established, approved SNM indication.

| Food and Drug Administration (FDA) approved | Not approved |
|---|---|
| Urgency urinary incontinence (UUI) | Bladder pain syndrome (offer selectively if coexisting OAB) |
| Urgency–frequency | Pelvic pain syndrome |
| Chronic NOUR | Neurogenic bladder (but pooled success comparable) |

### Test phase — selection decides success

- Every patient has a test. **Success = ≥50% improvement** in ≥1 diary parameter (frequency, voided volume, leaks) vs a 3–5-day baseline; retention — catheterisation reduced ≥50%, ideally none.
- **Percutaneous nerve evaluation (PNE)**: local anaesthesia → needle into **S3** by landmarks (≈9 cm above coccyx tip, 1–2 cm lateral; fluoroscopic cross-hair optional) → sensory then motor responses → temporary lead, external stimulator, 5–7 days. Positive → full implant in one session.

- **Staged implant**: stage 1 — permanent **tined quadripolar lead** into S3 under fluoroscopy, tunnelled extension, test up to 4 weeks → stage 2 at 2–4 weeks: generator in buttock. Failure → remove lead. Aim: responses on all four contacts at low amplitude. Prophylaxis: cefazolin + metronidazole (± meticillin-resistant *Staphylococcus aureus* cover).

| | **PNE** | **Staged** |
|---|---|---|
| Setting | Office, no incision | Theatre |
| Lead | Temporary, migrates → false-negative test | Permanent tined, precise |
| Test | 5–7 days | Up to 4 weeks |
| Conversion | Lower | Higher |
| NOUR | Not used | Used |

| **Root** | **Perineal/anal** | **Leg/foot** |
|---|---|---|
| S2 | — | Calf/foot external rotation |
| **S3 (target)** | Anal "bellows", perineal/genital sensation | **Great-toe flexion** |
| S4 | Perineal/anal response | None |

### Device and programming

- Pulse width 210 µs, 10–16 Hz, amplitude at sensory threshold. Reprogramming ≈3 times in year 1. Cycling stimulation equals continuous. **Bilateral not superior** to unilateral (randomised controlled trial [RCT]).
- Battery: non-rechargeable ≥**10 years**, rechargeable ≥15 years.
- **Magnetic resonance imaging (MRI)**: current devices conditional for whole-body MRI at **1.5 and 3 T**; at replacement remove all old components.

### Complications

- Revision ~3%, explantation ~9% at 2 years (ROSETTA); loss of efficacy; lead dislocation; infection.
- †Implant-site pain, unwanted stimulation/leg pain — managed by reprogramming, then revision; infection → explant.

## 3 · TIBIAL NERVE STIMULATION

- Posterior tibial nerve carries S2–S3 fibres (shared with bladder). Mechanism: cortical reorganisation; in animal OAB models inhibition is opioid-mediated (C fibres; blocked by naloxone).

**ALGORITHM**
Supine → 34-gauge needle ≈3–4 cm (three fingerbreadths) above medial malleolus, behind tibia, at 60–90° → ground pad same leg → 20 Hz, 200 µs, 0–10 mA → confirm: **great-toe flexion/toe fanning** (motor) or sole tingling (sensory) → **12 weekly sessions × 30 min** → taper → maintenance

- Indications: OAB refractory to or intolerant of drugs (office-based, no implant); safe in neurogenic OAB (multiple sclerosis, Parkinson disease, stroke); children failing bladder training and drugs.
- Success: ≥50% fall in urgency/UUI. **Relapse is likely without maintenance**; tapering sustains results to 3 years.
- Vs tolterodine: equal efficacy, fewer side effects. Better outcome without DO; poor mental-health score predicts failure.
- **Transcutaneous (TTNS)**: home surface electrodes; skin impedance limits amplitude → less effective than PTNS, cheaper.
- **Implantable tibial stimulators** (BlueWind, eCoin) remove the clinic burden; PTNS response does not guarantee implant response.

## 4 · OTHER TARGETS

- **Pudendal nerve** (S2–S4, Alcock canal): tined lead, posterior approach (perineal has more infection). Blinded crossover vs SNM: greater improvement, preferred by most; salvage after failed SNM. Not FDA approved.
- **Dorsal genital nerve**: purely sensory; acutely suppresses neurogenic DO in spinal cord injury (SCI) — suits **conditional** (on-demand) stimulation; limited by electrode fixation and site. Experimental.
- **Sacral dermatome (transcutaneous)**: raises capacity; cannot abort contractions acutely.
- **Intravaginal stimulation** for stress incontinence: safe, evidence of low quality.
- †**Intravesical stimulation**: catheter electrode in saline-filled bladder activates mechanoreceptor afferents to improve sensation and reflex contraction in neurogenic underactive bladder (incomplete lesions; afferent pathway must be intact); evidence limited.

## 5 · SACRAL ANTERIOR ROOT STIMULATION (BRINDLEY)

- Stimulates **anterior sacral roots** → bladder contraction (voiding), defecation, erection — in complete SCI.
- Stimulation cannot reduce DO → combined with **posterior (dorsal) sacral rhizotomy** of the same roots: abolishes reflex incontinence, raises capacity and compliance, protects kidneys, abolishes autonomic dysreflexia.
- Voiding works because the striated sphincter relaxes faster than the detrusor after each burst → †"post-stimulus voiding". †Cost of rhizotomy: loss of reflex erection, reflex ejaculation and residual sacral sensation.

## 6 · LANDMARK TRIALS

| Trial | Design | Result | Take-home |
|---|---|---|---|
| SUmiT | RCT, PTNS vs validated sham, 12 weeks, n = 220 | Responders 55% vs 21% | First sham-proven PTNS efficacy; maintenance keeps 77% relapse-free at 3 years |
| InSite | RCT, SNM vs standard medical therapy, mild–moderate OAB | Success 61% vs 42% at 6 months | SNM beats continued drugs |
| ROSETTA | RCT, onabotulinumtoxinA 200 U vs SNM, refractory UUI women | Toxin slightly fewer leaks at 6 months; equal at 2 years; toxin more UTI, some clean intermittent catheterisation | Both valid third-line; registered OAB toxin dose is 100 U |

## 7 · MCQ ONE-LINERS

1. SNM target root: S3 — bellows and great-toe flexion.
2. S2 response: calf/foot external rotation; S4: perineal response, no toe.
3. SNM test success: ≥50% improvement in a diary parameter.
4. Urodynamic DO does not predict SNM success.
5. FDA-approved SNM: UUI, urgency–frequency, NOUR.
6. Neurogenic bladder and pelvic pain: not FDA-approved for SNM.
7. Staged implant preferred for NOUR; PNE not used.
8. Bilateral SNM not superior to unilateral.
9. Modern SNM devices: conditional MRI at 1.5 and 3 T.
10. SNM in retention acts mainly by sphincter relaxation.
11. Fowler syndrome predicts good SNM response.
12. Current SNM theory: afferent modulation of forebrain awareness centres.
13. PTNS: 34-gauge needle, 3–4 cm above medial malleolus, 20 Hz.
14. PTNS protocol: 12 weekly 30-minute sessions, then maintenance.
15. PTNS derived from acupuncture point SP6.

6. First tibial nerve stimulation for bladder: McGuire, 1983.
7. SUmiT: first validated sham; 55% vs 21%.
8. PTNS inhibition of OAB-type reflex is naloxone-sensitive.
9. TTNS less effective than PTNS due to skin impedance.
20. Pudendal stimulation: salvage after failed SNM.
21. Brindley stimulator combined with posterior sacral rhizotomy.
22. ROSETTA used 200 U toxin — twice the registered OAB dose.

# 123 Surgical Procedures for Sphincteric Incontinence in the Male: Artificial Urinary Sphincter and Perineal Slings

*Reading time ≈ 25 min (slow pace)*

## 1 · PATHOPHYSIOLOGY AND DEVICE MECHANISMS

- Male **sphincteric (stress) urinary incontinence (SUI)** needs failure of **both** sphincters: internal (bladder neck, prostatic urethra) and external (rhabdosphincter). Radical prostatectomy (RP), the commonest cause, removes the internal sphincter by design, so continence rests entirely on the rhabdosphincter. Others: pelvic radiotherapy (RT), pelvic fracture urethral injury, neuropathy.
- Vesicourethral anastomotic stenosis (VUAS): its treatment can damage the membranous urethra and cause SUI.
- Work-up (cough-test grading, pad test, cystoscopy, urodynamics), conservative care and sling-versus-sphincter overview → *Ch 106*.

| Device | Mechanism | Site | Adjustable | Needs residual sphincter |
|---|---|---|---|---|
| Artificial urinary sphincter (AUS, AMS 800) | Circumferential compression, patient-cycled | Bulbar urethra or bladder neck | No (pressure chosen at surgery) | No |
| Advance / Advance XP | **Repositioning** and lengthening, no compression | Bulbar, transobturator | No | **Yes** |
| Virtue (quadratic) | Prepubic compression + elevation | Bulbar, four arms | No | Less dependent |
| Adjustable transobturator male system (ATOMS) | Cushion compression | Bulbar, transobturator | Yes — scrotal port | Less dependent |
| ProACT (Adjustable Continence Therapy) | Two para-urethral balloons, lateral compression | Bladder neck | Yes — scrotal ports | — |

- **AUS**: cuff (≈2 cm compression length), scrotal pump, pressure-regulating balloon (PRB) that sets cuff pressure. Squeezing the pump empties the cuff; it refills passively from the PRB †over a few minutes. Open cuff lowers resistance, so men without detrusor contraction can void by straining/Credé.
- Transobturator sling: raises leak point pressure **without obstruction** by elevating and elongating the bulb and rhabdosphincter.

> **KEY POINT** Every device either compresses the urethra or repositions a functioning sphincter. Trap: ATOMS and Virtue are "slings" but act by compression — only the Advance depends absolutely on a coapting residual sphincter.

## 2 · INDICATIONS AND DEVICE SELECTION

- Indication: **irreversible sphincter incompetence** with bothersome urinary incontinence (UI) after conservative care. Do not delay beyond **6 months** for severe/gravitational SUI, especially with a cystoscopic sphincter defect.
- Choice: severity, RT, prior urethroplasty, bladder function, cystoscopic coaptation, dexterity, cognition, life expectancy, preference.
- Severity (24-h pad weight): mild **<150 g**, moderate **<400 g**. AUS preferred if >250 g or after RT (MASTER trial → *Ch 106*).

| Stratum (American Urological Association/Society of Urodynamics, Female Pelvic Medicine & Urogenital Reconstruction [AUA/SUFU]) | Recommendation |
|---|---|
| Severe UI or **any RT** | AUS first-line; sling only with counselling |
| Moderate, no RT | AUS or male sling; adjustable balloons in specialised centres |
| Mild, no RT | Continue pelvic floor muscle training (PFMT), sling, possibly AUS |

- Counsel all: expect ~1 thin pad/day; AUS lifespan **7–10 years**, replaceable; failed slings mostly go to AUS.
- **Sling requirements**: mild–moderate SUI, contractile bladder (hypocontractile → AUS, as it cannot overcome added resistance), well-vascularised non-irradiated urethra, coapting sphincter on cystoscopy.
- **AUS requirements**: dexterity and cognition to cycle the pump, healthy urethra, adequate compliance.

| Absolute — need an alternative | Relative |
|---|---|
| Low-compliance bladder threatening kidneys; low capacity (frequency/urgency incontinence); urethral tissue too poor → clamp, catheter, bladder neck closure, diversion | Need for frequent transurethral work — bladder cancer, refractory VUAS, radiation cystitis (repeated instrumentation → erosion, infection) |

- Radiation cystitis: hyperbaric oxygen and **6 months** free of bleeding before AUS.

> **KEY POINT** Each device's contraindication mirrors the other's requirement.

## 3 · THE HIGH-RISK ("FRAGILE") URETHRA

- Fragile urethra = raised cuff-erosion risk. Strong factors: **prior RT, prior erosion/infection, previous urethroplasty**; also hypogonadism, catheterisation, 3.5-cm cuff after RT, tandem distal cuff. Debated: coronary disease, age >80, diabetes, smoking.
- RT: microvascular spongiosal injury; erosion earlier and commoner.
- Prior erosion: 5-year repeat erosion approaches **50%**. Urethroplasty: worse than RT — transection divides bulbourethral arteries (loses dual blood supply).
- Countermeasures: optimise medically, **transcorporal (TC) cuff**, PRB **51–60 cm $H_2O$**, cuff 0.5 cm larger than measured, nightly deactivation.

> **KEY POINT** Erosion = spongiosal ischaemia under constant pressure. Every risk factor reduces perfusion; every countermeasure adds tissue (TC tunica) or lowers pressure — never squeeze harder.

## 4 · OPERATIVE TECHNIQUE

### Preparation (all implants)

- General/spinal, low lithotomy, chlorhexidine–alcohol preparation. Urethral catheter optional (identifies urethra).
- Intravenous antibiotics within **60 min** of incision: aminoglycoside 5 mg/kg + first/second-generation cephalosporin or vancomycin. Sterile urine beforehand.

### Bulbar cuff AUS (perineal)

- Site: **proximal bulbar urethra, proximal to corporal fusion** — largest spongiosal diameter, safe circumferential dissection, cuff not opened by sitting; distal sites kept for revision.

**ALGORITHM**
Midline perineal incision → Colles fascia → bulbospongiosus divided → incise Buck fascia where it reflects off the urethra onto the diverging crura → 2-cm tunnel dorsal to Buck fascia over the urethral roof under vision → right-angle clamp → measure circumference → cuff (usually 4.0–4.5 cm) → tubing through bulbospongiosus deep to Colles fascia

- Sizing: a consistent method matters more than catheter in/out; cuff slightly tighter than tape; less tight cuffs continent better. Sizes 3.5–11 cm. **3.5-cm cuff** (atrophy, very distal) raises erosion after RT → TC and/or 51–60 PRB.
- **PRB**: abdominal (lower-quadrant incision → split muscle → preperitoneal pocket → close fascia round tubing) or via external ring (perforate inguinal floor, or ectopic submuscular above transversalis fascia). Avoid the blind ring route after mesh hernia repair or cystectomy. Fill **23 mL** saline or isotonic diluted contrast (not if iodine allergy).
- **Pump**: anterior scrotum, subdartos. Quick connectors.
- Cycle and urethroscopy: closed cuff → slight blanching; open → minimal coaptation (excess → larger cuff).
- Leave **deactivated 4–6 weeks** — essential for healing without erosion. †Deactivate the cuff (open) before any later urethral catheterisation or endoscopy.

| PRB (cm $H_2O$) | Use |
|---|---|
| 51–60 | High-risk bulbar (RT, prior erosion, atrophy) — trades continence for safety |
| **61–70** | Standard bulbar cuff |
| 71–80 | Bladder neck cuff |

## Variants

- **Tandem cuff** (incomplete occlusion; up to 11% leak with one cuff): new cuff ≥1.5–2 cm distal → join tubing with metal Y-connector → add 3 mL → deactivate 6 weeks. Not first-line — distal cuff erodes more, uses up revision sites.
- **Transcorporal cuff** (George Webster 2002 MCQ): parallel longitudinal tunical incisions on each corpus beside the spongiosum → dissect through intercorporal septum → ventral tunica stays on dorsal urethra → imbricate tunical edges → measure, place. For urethra <3.5 cm, revisions, prior erosion/urethroplasty (use a fresh site). Adds bulk to the thin dorsal spongiosum and avoids urethral injury; erections usually preserved; more transient retention; erosion benefit unproven.
- **Bladder neck AUS**: prostate intact (neuropathic, myelomeningocele); contraindicated after RP; open or robotic vesicoprostatic dissection; cuff ≥8 cm, PRB 71–80; less erosion and atrophy.

## Male slings

**ALGORITHM**
**Advance XP**: perineal incision → open bulbospongiosus → mark and divide perineal body 1–2 cm (releases bulb) → stab incision one fingerbreadth below adductor longus tendon → helical trocar outside-in (two "pops") onto fingertip at spongiosum–ramus angle → mesh tension-free, 4 absorbable sutures to spongiosum → tension arms equally under cystoscopy until mucosa coapts (bulb moves 3–4 cm proximally) → bury arms, layered closure

- **ATOMS**: bulbospongiosus **left intact**; needles round ischiopubic ramus; cushion under bulb; subdartos scrotal port, 9 mL; adjusted later in clinic.
- **ProACT**: fluoroscopic perineal trocars to bladder neck; balloons; scrotal ports.
- Advance is tensioned once, at surgery; adjustable devices titrate in clinic at the cost of a port and more complications.

## 5 · AUS COMPLICATIONS AND MANAGEMENT

### Retention

**ALGORITHM**
Confirm cuff deactivated → small urethral catheter 24–48 h → fails second trial → **suprapubic cystostomy** (ultrasound/fluoroscopy-guided, avoid PRB) → retention for weeks = undersized cuff → replace cuff

- Commoner with TC and 3.5-cm cuffs. Late retention → endoscopy + urodynamics.

### Infection

- **1–3%** primary, up to 10% high-risk; *Staphylococcus epidermidis*, *S. aureus*. Antibiotic-coated (InhibiZone) devices have not reduced AUS infection.
- **Scrotal pain = key early sign** (± erythema, pus). Antibiotics fail → explant all; reimplant 3–6 months.
- Immediate salvage (non-eroded): remove all → antiseptic irrigation → new device; not with sepsis, erosion, immunosuppression or gross pus.

### Urethral erosion

- **2–15%**; up to 35% with several risk factors. Remove **all components** (assume infected); loose closure/secondary intention. Post-erosion stricture 8–35%.

| Urethral loss | Repair |
|---|---|
| <50% | Catheter (~3 weeks), urethrogram before removal |
| >50% / complete | Unresolved: in situ ventral approximation (fewer strictures than catheter alone) or mobilisation + anastomosis; pseudocapsule closure does not prevent stricture |

- Reimplant at **3–6 months** after confirming patency, at a **new site**; TC ± low-pressure PRB; nightly deactivation. Repeat erosion up to 35%.

### Urethral atrophy

- Chronic spongiosal compression → **gradual** recurrent leak; a commonest cause of revision; cystoscopy shows incomplete coaptation.
- Downsize in place → if already 4.0 cm, move proximally, or distally with TC; or tandem. **Never simply raise PRB pressure** — erosion.

### Mechanical failure

- <3 years: replace failed part (pump or cuff). **>3 years: replace entire device** (PRB usual culprit; slow leaks hidden).

| Recurrent leak after AUS | Clue → confirm |
|---|---|
| Deactivated | Sudden; reactivate (squeeze pump edges / press button) |
| Fluid leak | Sudden; PRB shrinks on film/ultrasound |
| Atrophy | Gradual; PRB volume normal (>20 mL) → cuff options |
| Erosion / detrusor overactivity | Cystoscopy / urodynamics |

**KEY POINT** Tempo points to cause: sudden = device, gradual = urethra. Every complication is managed to keep a salvageable urethra for the next cuff.

## 6 · SLING COMPLICATIONS AND FAILURE

| Device | Commonest | Erosion / explant |
|---|---|---|
| Transobturator | Transient groin pain, wound issues; retention/urgency 4–10% | 0–0.4% — very rare |

| Device | Commonest | Erosion / explant |
|---|---|---|
| ATOMS | More complications | Erosion/infection ~10%; explant 10–16% |
| ProACT | **Perforation** (~5%); migration; revision 22% | ~4% |

- Cystoscopy in every worsening sling (mesh erosion). Infection/erosion → explant, AUS after 3–6 months.
- Failed sling → **AUS = gold standard**, results as for primary AUS; repeat Advance only if AUS refused (~56%).

## 7 · SPECIAL CIRCUMSTANCES

- Stricture at the cuff may herald erosion. VUAS proximal to cuff → balloon or holmium laser via flexible scope; large scopes → uncouple cuff.
- AUS + inflatable penile prosthesis (IPP): synchronous = metachronous; metachronous → AUS first.
- **Climacturia**: sling or AUS; with erectile dysfunction → **mini-jupette** graft at IPP.

## 8 · OUTCOMES

| Device | Outcome | Weak point |
|---|---|---|
| AUS | Social continence (0–1 pad) 55–77%; satisfaction ~90% | Revision 10–29% at 3 years |
| Advance XP | Cure ~60%, durable | RT, severe UI, prior surgery |
| ATOMS / ProACT | ~60–67% dry | Complications, limited data |

- Cure clusters at 55–65%; devices differ in durability, erosion and eligibility, not cure.

## 9 · MCQ ONE-LINERS

1. AUS = gold standard for all SUI severities.
2. Only Advance sling depends absolutely on residual sphincter.
3. Any prior RT → AUS first-line.
4. Detrusor hypocontractility → AUS, not sling.
5. Standard bulbar PRB: 61–70 cm $H_2O$.
6. Bladder neck PRB: 71–80 cm $H_2O$.
7. AUS activation delayed 4–6 weeks.
8. Transcorporal cuff introduced by Webster, 2002.
9. Bladder neck AUS contraindicated after RP.
0. Strong erosion risks: RT, prior erosion, urethroplasty.
11. Repeat erosion after prior erosion approaches 50% at 5 years.
12. Early AUS infection sign: scrotal pain.
13. Atrophy: downsize cuff, never raise PRB pressure.
14. Mechanical failure >3 years: replace whole device.
15. Retention after AUS: suprapubic cystostomy after second failed trial.
16. Commonest ProACT complication: perforation.
17. PRB fill volume: 23 mL.
18. Mild SUI: <150 g/24 h.
19. Erosion: explant all components.
20. Failed sling → AUS.

# ★ Putting it together: voiding dysfunction decisions

*Reading time ≈ 8 min (slow pace) · Integrates Ch 102–115 · use these as the skeleton of long-essay answers*

## A · THE FUNCTIONAL CLASSIFICATION AS A TREATMENT MAP

| | **Because of the bladder** | **Because of the outlet** |
|---|---|---|
| **Failure to store** | Overactivity or low compliance → antimuscarinic, β3-agonist, onabotulinumtoxinA, neuromodulation, augmentation | Sphincter weakness → pelvic floor training, duloxetine (Europe), sling, artificial urinary sphincter, bulking |
| **Failure to empty** | Underactive or acontractile detrusor → clean intermittent catheterisation (CIC); sacral neuromodulation for non-obstructive retention | Obstruction (anatomical, or dyssynergia) → α-blocker, prostate/bladder-neck surgery, sphincterotomy, CIC |

If neither can be fixed, circumvent: CIC, indwelling catheter or urinary diversion (→ *Ch 104*).

## B · WHERE IS THE LESION? EXPECTED URODYNAMICS

| **Lesion** | **Detrusor** | **Striated sphincter** | **Key risk** |
|---|---|---|---|
| Above the pons (stroke, Parkinson disease, dementia) | Overactive | Synergic | Urgency incontinence; low upper-tract risk |
| Spinal cord between pons and sacral cord (spinal cord injury, multiple sclerosis) | Overactive | **Dyssynergic** | High pressure → upper-tract damage; autonomic dysreflexia if lesion above T6 |
| Sacral cord / cauda equina / peripheral (diabetes, pelvic surgery) | Areflexic, often reduced sensation | Fixed tone, cannot relax voluntarily | Retention, overflow; may lose compliance |

**KEY POINT** Dyssynergia appears only when the lesion lies between the pons and the sacral cord, because the pons coordinates detrusor contraction with sphincter relaxation. A detrusor leak point or storage pressure ≥40 $cmH_2O$, or poor compliance, threatens the kidneys and is the main reason to treat (→ *Ch 107, 109*).

## C · OVERACTIVE BLADDER: TREATMENT LADDER

**ALGORITHM**

**1.** Behavioural: fluid and caffeine adjustment, bladder training, pelvic floor muscle training; treat constipation; review drugs.

**2.** Drugs: antimuscarinic or β3-agonist (prefer β3-agonist or a low-CNS antimuscarinic in the elderly); combine if partial response; check post-void residual in men with obstruction (→ *Ch 113*).

**3.** Refractory: intradetrusor onabotulinumtoxinA (100 U idiopathic, 200 U neurogenic; warn about CIC), percutaneous tibial nerve stimulation, or sacral neuromodulation (staged test; success = ≥50% improvement) (→ *Ch 115*).

**4.** Last resort: augmentation cystoplasty or urinary diversion.

## D · SPINAL CORD INJURY: MANAGING THE BLADDER

**ALGORITHM**

**Spinal shock** (areflexic bladder): indwelling or intermittent catheter until reflexes return.

**Then** urodynamics; CIC + antimuscarinic for a high-pressure overactive bladder; onabotulinumtoxinA if needed; sphincterotomy or diversion if CIC is impossible; lifelong upper-tract surveillance.

**Autonomic dysreflexia** (lesion above T6): sudden hypertension, bradycardia, headache, flushing above the lesion → sit upright, loosen clothing, find and remove the stimulus (blocked catheter, full bladder, constipation), short-acting antihypertensive if pressure stays high (→ *Ch 109*).

## E · NOCTURIA: THREE QUESTIONS FROM THE BLADDER DIARY

**ALGORITHM**

**1. Global polyuria?** 24-h output >40 mL/kg → look for diabetes mellitus or insipidus, polydipsia — always investigate.

**2. Nocturnal polyuria?** Night fraction >20% (young) or >33% (older adults) → heart failure, sleep apnoea, oedema states, evening fluids; consider desmopressin with sodium monitoring.

**3. Reduced night capacity?** Nocturnal bladder capacity index >1.3 → overactive bladder, obstruction, incomplete emptying; treat the lower urinary tract. Screen sleep disorders in all ("Sleep CALM") (→ *Ch 112*).

## F · MALE STRESS INCONTINENCE AFTER PROSTATECTOMY

**KEY POINT** Pelvic floor training first, but do not delay surgery beyond about 6 months for severe leakage. Grade by 24-h pad weight (mild <150 g, moderate <400 g). Mild–moderate leakage, contractile detrusor, no radiotherapy → male sling; heavy leakage (>250 g), any radiotherapy or a failed sling → artificial urinary sphincter (cuff, pressure-regulating balloon, scrotal pump). Erosion follows spongiosal ischaemia — risk rises after radiotherapy, previous erosion or urethroplasty; counter it by adding tissue (transcorporal cuff) or lowering pressure, never a tighter cuff. † An eroded or infected device is removed (→ *Ch 106, 123*).